



Quarterly Financial Disclosure Statement
Three months ended March 31, 2024

Allina Health System



Minnesota United's power soccer team, which features members of the Allina Health Courage Kenny Power Soccer team, showcased their skills outside Allianz Field in the summer of 2023.

For additional information please visit allinahealth.org.
Direct questions regarding disclosure information to treasury@allina.com.

TABLE OF CONTENTS

Page

OFFICER’S CERTIFICATION	2
OVERVIEW OF ALLINA HEALTH	3
ALLINA HEALTH SYSTEM – CONSOLIDATED FINANCIAL STATEMENTS.....	24
ALLINA OBLIGATED GROUP – COMBINED FINANCIAL STATEMENTS.....	33
ALLINA HEALTH SYSTEM – CONSOLIDATING FINANCIAL STATEMENTS	36
CONSOLIDATED UTILIZATION	39
MANAGEMENT’S DISCUSSION AND ANALYSIS OF RESULTS OF OPERATIONS	40
DEBT AND INVESTMENT APPENDIX	45

Officer's Certification

I, the undersigned, being the Interim Chief Financial Officer of Allina Health System (Allina Health), a Minnesota non-profit corporation, do hereby certify and declare that the accompanying interim consolidated financial statement of the Allina Health System and the Allina Obligated Group for the three months ended March 31, 2024, have been prepared in reasonable detail and are accurate in accordance with generally accepted accounting principles applied on a basis consistent with the accounting practices reflected in the annual financial statements.

These interim financial statements were developed by Allina Health and were not audited by an independent accounting firm.

Dated: May 15, 2024

By: /S/ Doug Watson

Doug Watson
Interim Chief Financial Officer
Allina Health System

Overview of Allina Health

Introduction

Allina Health System, primarily doing business as Allina Health (“Allina Health”), is a Minnesota non-profit corporation and an integrated health care delivery network primarily serving the communities in the greater Minneapolis-St. Paul (the “Twin Cities”) metro area and the western part of Wisconsin. Allina Health is anchored by three Twin Cities metro area hospitals, Abbott Northwestern Hospital (“Abbott”), United Hospital (“United”) and Mercy Hospital (“Mercy”), and is supported by a network of community hospitals, including two critical access hospitals. Allina Health is staffed by employed physicians and a large, aligned provider network. With more than 28,000 full- and part-time employees as of December 31, 2023, Allina Health is one of the largest employers in Minnesota. Allina Health consolidated revenue for the three months ended March 31, 2024, was \$1.3 billion.

The Allina Health Mission, Purpose and Promise

A Deep Commitment to Mission

Allina Health stands firmly on its mission as a nonprofit health care provider:

Allina Health serves communities by providing exceptional care to prevent illness, restore health and provide comfort to all who entrust the organization with their care.

Purpose and Promise

Building on Allina Health's strong nonprofit mission, the organization is taking steps to further solidify a mission-driven culture through the declaration of a purpose and promise. Through the activation of the organization's purpose and promise, Allina Health will build a strong and differentiating culture able to take on the headwinds facing non-profit health care in a successful and sustainable way.

Our Purpose | *Allina Health exists to enable the flourishing of humanity*

President and Chief Executive Officer Lisa Shannon declared Allina Health's flourishing purpose in early 2023. Allina Health and the health care industry have been through many unique challenges exacerbated by the COVID-19 pandemic and its ongoing impact. Looking ahead to the future of health care, it's clear that recovery and healing is needed for both those the organization serves and for those who serve a caring mission. Allina Health is creating a culture anchored in the flourishing of its team, patients, communities and the planet. To flourish, a person must feel that their whole self is valued, heard and supported. Allina Health seeks to create a sense of belonging so that each person can flourish in ways that are unique and meaningful to them.

Our Promise | All Together Better

Flourishing is not articulated through a medical diagnosis. It comes down to how patients feel. Allina Health's Promise captures how it should feel to work or receive care at Allina Health in an environment that fosters flourishing. When the organization's people and communities flourish, they should feel *All Together Better*. At Allina Health, All Together Better has a multi-dimensional meaning. It requires the full spectrum of services and teams to provide better care to those we serve. Health is about more than individuals, it's about the health of whole communities. By approaching care like this, better outcomes are achieved. Allina Health's *All Together Better* promise is a commitment to keep striving for a world in which all people and all communities flourish.

Mission in Action

As a health care organization committed to improving health and preventing illness, Allina Health recognizes that health goes beyond a clinical diagnosis. In support of Allina Health's commitment to delivering holistic care, the organization seeks to identify unmet health-related social needs in the patients and communities Allina Health serves through the Health-Related Social Needs Program. The program is an evolution of the Accountable Health Communities Model, a five-year cooperative agreement with the Centers for Medicare and Medicaid Services (CMS) that demonstrated how identifying and addressing health-related social needs impacts health care quality, utilization and costs. Allina Health offered and completed more screenings than any other participating organization in the country, according to CMS, and now, the organization is transforming this work with the Health-Related Social Needs Program. The new program screens patients more efficiently and effectively to understand how social isolation, food insecurity, housing instability, difficulty paying utilities, access to transportation and safety make a difference in every patient's health. The program screened more than 393,000 patients in 2023 across all clinic locations and admitted and observation patients across the system. With that information in hand, Allina Health care staff are able to connect patients with resources built to alleviate or address these needs. For example, patients experiencing urgent food needs at an Allina Health clinic may receive bagged meals for immediate support as well as a connection to a community resource to address food insecurity.

Allina Health by the Numbers

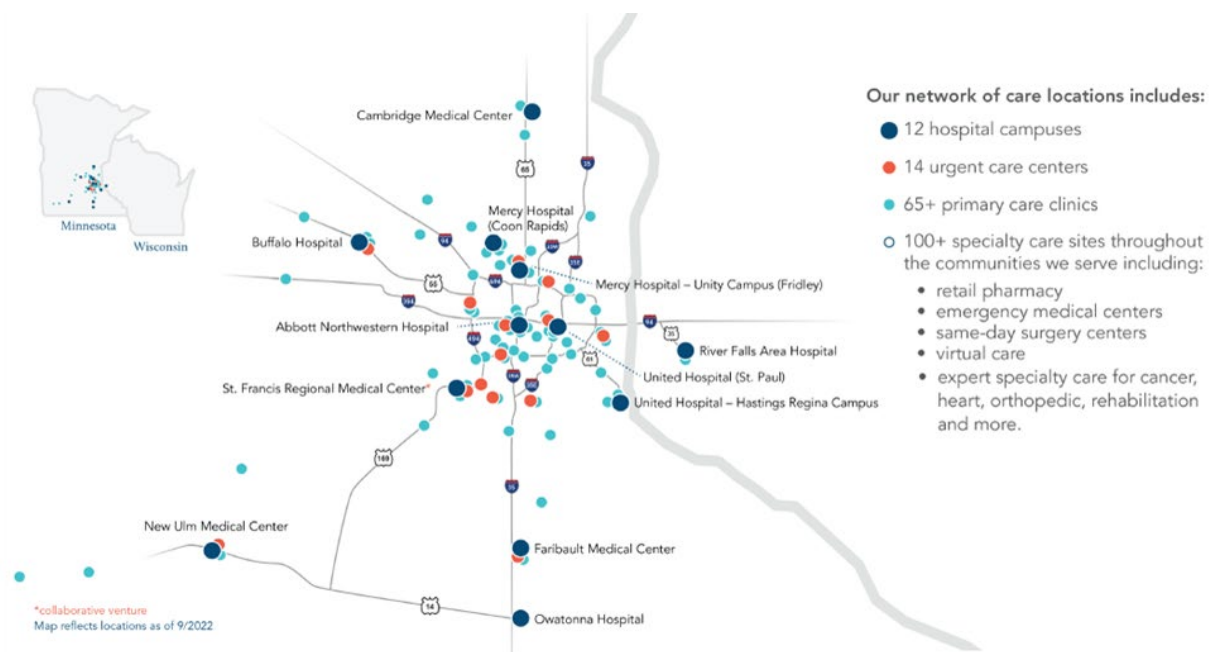
Allina Health owns and operates 11 hospital campuses and jointly owns and operates one other hospital. These include urban tertiary care, suburban community, and rural hospitals. Allina Health hospitals provided 24,016 inpatient admissions and 298,957 outpatient visits during the three months ended March 31, 2024. As of March 31, 2024, Allina Health hospitals had a licensed bed capacity of 2,451 acute care beds, 1,851 of which were staffed for inpatient services.

Allina Health provides clinical services with care delivered by expert providers through numerous market-leading clinical service lines and institutes. More than 600 primary care physicians provide care across Minnesota and western Wisconsin by way of 60+ primary care clinic locations, as well as through virtual visits. For more specialized care needs, Allina Health employs and contracts with a wide range of specialists and sub-specialists, including but not limited to, cardiologists, neurologists, oncologists, orthopedic and sports medicine specialists, mental health and addiction, spine surgeons, general surgeons, pulmonologists, OB/GYNs, dermatologists and integrative health. In addition, hospital-based practices, such as hospitalists and intensivists help support the hospital campuses Allina Health operates.

Allina Health physicians and allied professionals generated 2,703,203 work RVUs during the three months ended March 31, 2024.

The Allina Integrated Medical (“AIM”) Network aligns Allina Health physicians, 1,500 independent medical physicians, and more than 20 hospitals to deliver market-leading quality and efficiency in patient care. The AIM Network helps power Allina Health as a comprehensive health care system with one of the largest physician networks in Minnesota.

The AIM Network is structured to contract with third-party payers under outcomes-based payment arrangements. The AIM Network is organized to provide a narrow network of Allina Health-aligned providers to share quality and cost-performance risk. The AIM Network has developed a set of performance measures and data-sharing mechanisms to enhance its performance under outcomes-based payment arrangements. One such arrangement that includes one of AIM Network’s Accountable Care Organization products is Metro MN, which partners with Blue Cross Blue Shield of Minnesota to optimize value-based care with strong capabilities, such as care management, and lower per-member per-month costs compared to market benchmarks.



A Leading Health Care Organization in the Region

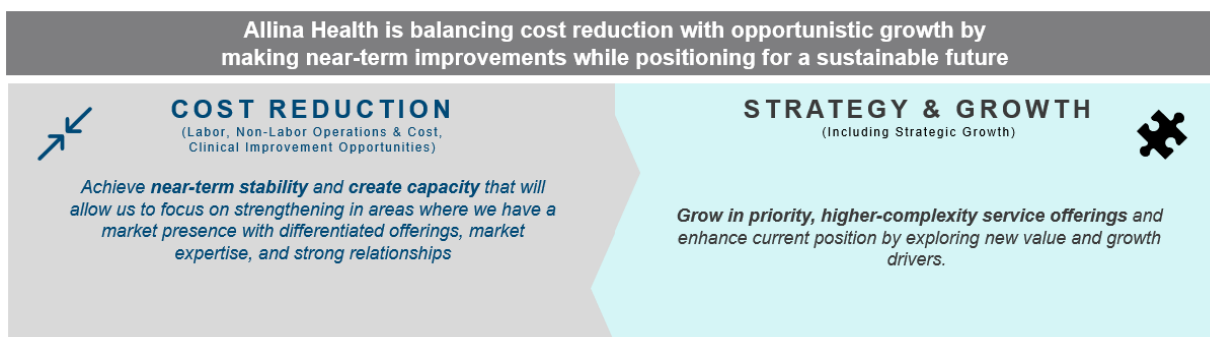
Allina Health has a strong market position in key clinical service lines relative to its competitors. Allina Health's main strengths are in cardiovascular and neuroscience, with strong market competitive services in orthopedics. At Allina Health, patients have a go-to health care partner with convenient access to expert primary care providers at more than 60 primary care clinics. For more specialized needs, patients are connected with expert care teams from service lines like Allina Health Neuroscience, Spine and Pain Institute, which has quickly become a regional leader in complex care by aligning, coordinating and building upon the extensive talent and clinical programs across the system to create a patient-focused approach to care that supports a patient from disease prevention to quaternary care and post-acute to home care. Additionally, Allina Health Cancer Institute provides the seamless, comprehensive, whole person cancer care patients need, serving as a more connected alternative to the fragmented nature of oncology services in the Twin Cities.

Allina Health continues to have a stable and leading market share in the metropolitan area, with a 31.1% inpatient market share as of September 30, 2023.

As an integrated health care system that includes hospitals, employed physicians, emergency medical services, ambulatory care, pharmacy services, home care, hospice services and an automated electronic medical record system, Allina Health is a leader in health care in the Twin Cities area and greater Minnesota, and is well-positioned for the continued evolution of health care delivery.

Performance and Business Transformation

Allina Health has been steadfastly committed to transforming the organization's performance and business as nonprofit health systems across the country have navigated a confluence of negative economic and market factors including staffing shortages, increased cost of goods and services, declining reimbursements relative to expenses incurred, investment portfolio losses, supply chain challenges and other issues. The organization's focused response to these pressures began to show significant results in the final quarter of 2023 after experiencing operating and investment losses in 2022 and operating losses in 2023. This performance improvement is an important building block that empowers a continuation of this journey to achieve the goal of posting a breakeven operating margin in 2024 by accelerating growth and cost improvement while structuring the organization for today's volatile health care environment.



Performance Transformation

Allina Health has partnered across the care continuum for years. In the post-pandemic environment, Allina Health has redoubled its efforts at performance transformation, partnering with market-leading collaborators which are dedicated to accelerating the system's population health efforts and enabling appropriate and timely discharge to post-acute care. Additionally, as of April 1, 2023, Allina Health has partnered with Huron Consulting Group, a nationally-recognized consultant, to support the organization's efforts to achieve key performance initiatives—including optimizing patient flow, labor productivity, fixed and variable labor benchmarking to ensure quality and efficiency, corporate overhead structures and associated administrative expenses, revenue cycle management and assessing optimal footprint to deliver on key care delivery strategies. In conjunction, Allina Health is developing in-house solutions where

appropriate to improve performance and create a better experience for patients and teams faster than would otherwise be possible to support patients and the community.

Allina Health's performance transformation requires a dual approach that focuses both on strategic growth and cost reduction to ensure sustainability. The organization remains focused on providing the most needed services to the communities we serve in an efficient and sustainable care delivery model.

Since the beginning of April 2023, Allina Health has made encouraging progress in transforming performance through numerous key initiatives including reducing spending on agency staff, reducing discretionary spending and more. Allina Health has made a concerted effort to expand access to skilled nursing facilities, reduce delays in patient transfers when appropriate and, ultimately, ensure patients receive the right level of care at the right place. This creates a better patient experience and improves access throughout the system. Since the start of April 2023, Allina Health decreased the average length-of-stay from 5.2 days to an average of 4.9 days for 2023.

Workforce planning remains a key focus for Allina Health as the organization works to create innovative pathways and development opportunities to ensure patients receive care from Allina Health care teams, improving outcomes and reducing the need for agency resources. One recent example is Allina Health's new agreement with the University of Minnesota School of Nursing that will develop new strategies for nursing education, recruitment and professional Development (see "Workforce Planning" below).

Business Transformation

The region, like others, has seen an increase in proposed joint ventures, partnerships and mergers designed to adapt and strengthen organizations in the face of market headwinds. Like others, Allina Health routinely explores vertical and horizontal partnerships, joint ventures, mergers, combinations and other affiliations with similarly mission-driven people and organizations, including hospital systems, independent physician groups, payers, third-party consultants and managers, technology companies, retail entities, public or academic entities, investors and venture capital and other providers of clinical and administrative services, both regional and national. Partnerships may include divestitures or outsourcing of significant lines of business in compliance with applicable covenants.

Allina Health has developed a set of inviolable principles that guide consideration for potential partners. These principles will be a lens for leadership to use as they consider potential partners and whether those partnerships will be worth further analysis. These principles are meant to be broad, but also reflect a clear distillation of priorities. The principles are the following:

- Align culturally and strategically to improve care for Allina Health's communities.
- Ensure communities across Allina Health's footprint will continue to have access to high-quality, affordable, patient-centered care.
- Deliver continued commitment to excellent care.
- Yield demonstrably greater value than Allina Health could achieve on its own.

Changing Health Care for the Better: Our Five-Year Strategic Plan

A clear strategic focus is crucial to effectively serving patients and communities in a rapidly evolving health care marketplace. Recent industry dynamics, including the COVID-19 pandemic, inflation and staffing challenges have only strengthened the organization's strategic resolve and emphasized the priorities set in place. Allina Health's strategy is more important than ever and proves a need to accelerate the implementation of key initiatives. The headwinds in the health care industry will always change, and leadership remains confident that the organization's strategic focus will not only guide the organization through challenging times but also position Allina Health for continued success in the future. Allina Health's strategy is enabled by:



- Investing in Allina Health's people and culture
- Delivering a seamless and inclusive experience
- Achieving clinical excellence
- Driving operational excellence
- Payer partnerships
- Diversifying the organization's sources of revenue

The key enablers are critical to transforming the enterprise on multiple, interwoven dimensions. Allina Health is evolving from a holding company – created through acquisitions and organic growth over decades – to a highly aligned operating company by strengthening the organization's functions and systemic capabilities. Allina Health is moving from primarily responding to illness to a more proactive approach as a high-value complex care coordinator by developing multi-channel access to care and engaging with the community and consumer. This shift in mindset enables Allina Health to address patient health concerns early and empower patients to live their healthiest lives.

Allina Health believes that investing in these imperatives will enable the organization to deliver high-value care in an integrated, coordinated and seamless fashion to patient populations increasingly defined by complex clinical and non-clinical issues.

Minnesota and western Wisconsin communities depend on Allina Health. With that dependency comes a responsibility to grow and adapt to the needs of patients and communities Allina Health serves, and to be more accessible while improving health care delivery. Allina Health's strategy will transform the organization's ability to deliver safe, high-quality care while creating a better experience for patients and enabling Allina Health to do so for generations to come.

Clinically Integrated Care Delivery Model

In October 2023, Allina Health implemented a new Clinically Integrated Care Delivery Model that brings together acute and ambulatory care under a unified leadership structure. This new structure sharpens the organization's focus on a patient's journey throughout the care continuum, allows leaders to co-create and co-lead, elevates the talents and expertise of providers, and seeks to break down silos and create opportunities for continuous improvement.

The new model takes a regional approach to care delivery with strong clinical leadership across all settings and systemwide support services. Dave Slowinske serves as President, Northeast Region with Robert Quickel, MD, vice president, Surgical Services, serving as his dyad partner, overseeing Mercy Hospital, United Hospital, Buffalo Hospital, Cambridge Medical Center, River Falls Area Hospital and many clinics in that geographic region. David Joos serves as President, Southwest Region with William Katsiyannis, MD, president, Minneapolis Heart Institute, serving as his dyad partner, overseeing Abbott Northwestern Hospital, New Ulm Medical Center, Faribault Medical Center, Owatonna Hospital and many clinics in that region. Bill Evans serves as Vice President, Ambulatory

Operations, supporting primary care, specialty care and urgent care clinics across both regions as well as ambulatory surgery centers.

Expanding and Growing Access to Value-Based Care

Allina Health is dedicated to whole person-centered health care, standing on the organization's vibrant, pioneering and community-driven legacy to deliver on Allina Health's purpose: the flourishing of humanity. Allina Health has earned the trust of individuals and communities, care teams and employees through innovations that enhance the care model across the care continuum with a focus on easy access, affordability, quality and safety for all people.

To support the health of its patients and communities, Allina Health is committed to delivering high-value care and performing on outcomes-based payment models. As trusted partners in care, Allina Health walks alongside individuals during their care journey to eliminate barriers and create a seamless health care experience. Expanding care management services, using data to inform best practices and collaborating with partners are key to transforming Allina Health's business as well as patient care. Additionally, Allina Health is working to evolve the current reimbursement model so that it can be reimbursed for services that matter to its patients and help make care more efficient, effective and affordable.

As part of this strategy, Allina Health has entered into contracts with health plan insurance payers that reward enhancing quality, improving outcomes, addressing health disparities and reducing total cost of care. The contracts incentivize preventive and coordinated care to foster more proactive care and catalyze work to reduce the unacceptably high rates of health disparities across the community that we are working collectively and independently to eliminate. The organization has successfully built up the capabilities required to perform in quality and cost outcomes by advancing our ability to manage risk. Allina Health continues to expand its outcomes-based risk contract portfolio in a measured fashion to gain experience, manage risk factors within the organization's sphere of influence and maintain a focus on providing value more broadly. Allina Health partners with payers who are truly invested in transforming and innovating care models and paying for outcomes while reducing administrative burden to all teams so they can spend more time delivering expert care. Allina Health is implementing a phased approach that aligns with the goal of pursuing outcomes-based risk contracts that build scale over time. Allina Health's commitment to high quality, affordable care, equitable care outcomes and public health has resulted in care innovation, better health outcomes and public policy successes for the community it serves.



The Allina Health Cancer Institute is one way Allina Health is furthering seamless, comprehensive, patient-focused care.

Allina Health built one of the nation's highest-performing health-related social needs programs, screening more than 617,000 patients in two years. All patients who request support for their needs have been navigated to appropriate Allina Health or community-based resources, yielding over a 55% need resolution rate at re-screening. Using the health-related social needs screening data, the organization has successfully collaborated with partners to address individual and community social determinants of health. Payer partners have provided funding for the launch of this program and continue to partner on innovative solutions to reduce identified health disparities in the community.

Allina Health's value-based care journey is expected to result in better outcomes, lower premiums and improved access to care for patients while increasing the number of people in the community Allina Health serves and improving satisfaction for patients and provider teams. Also, health plans will grow and retain membership by offering competitively priced products and improved benefits to patients and employers. In an environment that requires health care organizations to be innovative and flexible, Allina Health's commitment to enabling population health, advancing payer partnerships and delivering whole person health to all people has caused positive outcomes and pioneering care models for the collective communities we serve.

Allina Health and Aetna Health Plan

In January 2017, Allina Health and Aetna signed an agreement to create a jointly owned health plan company, Allina Health and Aetna Insurance Company (“AHAIC”), which provides an innovative health care option for employers and consumers in the greater Twin Cities area. This jointly owned health plan is intended to fully align the incentives and capabilities of a national insurer and major local health system in ways Allina Health expects to lead to improved health outcomes and cost management. The ownership structure for the partnership is designed to streamline the patient experience by combining the power of insurer and provider data, coordinating integrated care teams and providing health insurance benefits and administrative services. Allina Health’s investment in AHAIC is expected to create returns both from a financial and operational perspective.

The Minnesota Department of Commerce issued a Certificate of Authority for AHAIC in August 2017, which made AHAIC an officially licensed health insurance provider and was the first new insurance company to enter the Minnesota market in many years. AHAIC began offering large group (51 or more members) products in 2018 and began to offer a Medicare Advantage product in 2019. As of December 31, 2023, AHAIC total membership was approximately 40,000, including approximately 19,000 Medicare Advantage members.

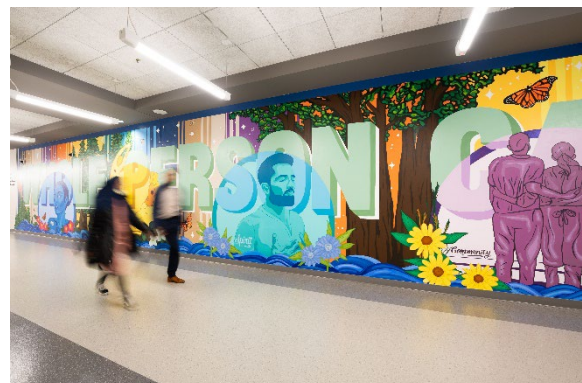
Driving Care Model Innovation

Allina Health is driving care model innovation to accommodate the needs of an ever-changing population. To do so successfully, Allina Health is focused on creating a patient-centric care model, with care management enabling integration across diagnosis, treatment and clinical support. Analytics and technology will improve the quality of care, lower the cost of care and increase accessibility while advancing care innovation through design-thinking and evidence-based care delivery.

Commitment to Safety and Quality. Safety and Quality are paramount to any care model and remain a key point of focus as Allina Health innovates. Allina Health’s commitment centers on preventing harm and encouraging a culture where all employees speak up and feel empowered. Allina Health has committed to a journey to zero preventable harm by adopting the successful systems of high-reliability organizations. Key components of this journey include extending multi-tiered safety huddles across the system and engaging in a systemwide program that will provide each employee and provider with the language, tools and standardized processes to respond to and prevent harm. To date, more than 88% of all Allina Health employees and 84% of all Allina Health providers have been trained in high reliability skills. In addition to training, Allina Health has invested in sharing safety event learnings and actions throughout the organization to prevent recurrence as well as the Safety Champion Program, supporting informal leaders in clinical and non-clinical departments to provide education and coaching on safety behaviors, allowing for a proactive approach to safety. Since 2019, Allina Health reduced serious safety events by approximately 50% in the pursuit of zero preventable harm.

Care Coordinator. As a system, Allina Health strives to be a high-value, complex care coordinator for its patients and communities. Solving consumer issues, including the growing gap between consumer expectations and actual experience, also supports Allina Health’s future market role as a care coordinator by seamlessly connecting all the necessary services for a patient’s unique needs. To address this imperative, Allina Health is pursuing both enterprise-wide tactics and building specific capabilities to pursue a better consumer experience through:

- A next-generation approach to consumer experience design, including designing its cancer infusion centers and new clinics in partnership with patient advisors.
- Centralizing and investing in key points of care connection, such as the Customer Experience Center and Access Center, which help create seamless connections for all patients across the care continuum.
- Offering non-traditional services based on consumer demand and preferences, including improved telemedicine capabilities and Acute Hospital Care at Home (see “Innovation” below).



- Building out a portfolio of innovative products and services that align with Allina Health's vision and principles.
- Prioritizing person-centered care with patients at the heart of an innovative care model.
- Educating and training staff on culturally appropriate practices, including gender-affirming care.
- Addressing disparities in key health outcomes, including increasing colorectal cancer screening rates and improving diabetes management for Black/African-American patients.

Innovation. Innovation is a key strategic initiative for Allina Health to identify alternative and emerging business opportunities to serve the patient, advance Allina Health's organizational strategy and mission, and positively impact Allina Health's operating results. Allina Health has identified key trends that will be a focus for innovation, driving at new care models, capabilities and consumer products.

One such innovative effort is Allina Health's Acute Hospital Care at Home program. Launched during the height of the COVID-19 pandemic, Acute Hospital Care at Home is a new program designed to decrease length of stay and total cost of care, reduce readmissions, increase patient satisfaction and divert patients from the emergency department or urgent care. By bringing patient care to the home, Allina Health created capacity within hospital resources while bringing care directly to the patient. Following the success of the program, Allina Health has partnered with Flare Capital Partners to create Inbound Health, a new company that increases access to hospital-at-home and skilled nursing-at-home programs by scaling the model nationally. The company combines biometric monitoring, digital surveillance, in-home nursing and therapy, virtual visits with hospitalists and geriatricians and a comprehensive supply chain to deliver a safe and high-quality care program.

Allina Health's virtual care services have continued to be a valuable option for patients following the exponential increase in demand with the start of the COVID-19 pandemic, totaling more than 470,000 virtual visits completed in 2023 alone. Virtual care services are often an entry point for patients into the Allina Health system and show strong trends of creating pathways for new attributed lives for the system. Allina Health's suite of virtual care services have grown as consumer expectations have shifted to seek alternatives to in-person care and technology has improved, improving access, enhancing experience and lowering cost of care. Allina Health's virtual care options include:

- scheduled virtual visits
- on demand virtual care
- online questionnaire-based virtual visits
- and more.

Scheduled virtual visits are offered by most ambulatory providers at Allina Health, including providers in the primary care, mental health, orthopedics, oncology and cardiology departments. Patients may also access on demand virtual care options via Virtual Urgent Care, where patients can connect directly with urgent care providers. In December 2023, Allina Health introduced on demand virtual orthopedic urgent care, the first offering of its kind in the market. Virtual urgent care is now available to Allina Health patients 24/7, in all 50 states. Allowing for maximum patient flexibility, patients may also access an asynchronous questionnaire-based visit, 24/7, and can expect provider responses within one hour, allowing patients to quickly get their medical needs met and to get back to their busy lives.

Allina Health has also made significant investments in its digital platform, empowering its patients with the information to make decisions and interact with its providers. Allina Health's patient portal includes Patient Wisdom, a guided online platform for patients to share stories about themselves, their health and their care with their Allina Health care team through integration with the electronic health record. Relative to the national health care market, leadership believes that Allina Health's digital capabilities are leading-edge, with strong offerings in digitized self-service like online bill pay through a partnership with Cedar as well as online scheduling and patient education.

Allina Health has dedicated \$80 million since 2017 to make investments in strategic management directed venture funds. The primary objectives of strategic management directed funds generally include increasing awareness into future market opportunities and trends, accessing a pipeline of vetted disruptors that will align with Allina Health's strategy to deliver enhanced value and innovation, and gaining exposure to products and services that could accelerate disruption and drive performance. Building from these activities, Allina Health now has strong relationships with the general partners and their strategic limited partners and in August 2022 launched its own Strategic Direct Investment Program. This program allows for direct co-investment into select investments that meet the following objectives: the investment opportunity accelerates a key strategic initiative associated with Allina Health's long-term Strategic Plan;

there is an opportunity for Allina Health to operationally collaborate with the direct investment company; a top-tier venture fund has performed extensive due-diligence and valuation of the investment to determine strong financial return potential for Allina Health. The first Strategic Direct Investment Program opportunity is KeyCare, an early-stage virtual care company designed to increase access, convenience and affordability of select health care service lines to communities. In March 2023, Allina Health and KeyCare began offering on demand 24/7 virtual urgent care. Allina Health expects to grow this strategic program in additional areas of Allina Health's strategic priority.

Optimizing Allina Health's Footprint

Optimizing Allina Health's footprint is core to the organization's ability to expand access to value-based care and lower total cost of care. To support this effort, Allina Health has enhanced multi-channel access to care in a variety of ways, including expanded virtual services and the introduction of the nationally recognized Home Hospital Care program which delivers safe, high-quality care in the comfort of the patient's home. Ambulatory surgery centers are an increasingly important offering in Allina Health's care offerings, enabling efficient, cost effective, high-quality care that is convenient for patients (see "Other Recent Initiatives and Developments – Ambulatory Surgery Center Strategy" below). Furthermore, Allina Health has focused on delivering complex care services, highlighted by the creation of the Allina Health Cancer Institute, which redefines traditionally fragmented cancer care to create a program that delivers comprehensive, expert, person-centric cancer care from diagnosis to survivorship (see "Other Recent Initiatives and Developments – Allina Health Cancer Institute" below). Allina Health has integrated mental health and well-being into primary care practice to improve Allina Health's ability to deliver excellent patient-focused care and support one of the highest needs in the community. That support has only increased with the creation of the Center for Integrated Mental Health and Addiction Care, a recently opened facility that transforms Allina Health's mental health and addiction services by offering the entire care continuum in one convenient location (see "Other Recent Initiatives and Developments – Center for Integrated Mental Health and Addiction Care" below). True integration of Allina Health's service lines and business units also requires unified workstreams and workforce. Allina Health is continually improving cohesion across all departments.

Diversity, Equity, Inclusion and Belonging and Sustainability

Allina Health has made significant strides in building the framework to make Diversity, Equity, Inclusion and Belonging (DEI&B) a priority for the organization. Following the murder of George Floyd, which occurred blocks from Allina Health's headquarters and Abbott Northwestern Hospital, Allina Health has recommitted to taking an active role in supporting the South Minneapolis community, including those who have long faced discrimination.

Allina Health's CEO Diversity, Equity, Inclusion and Belonging Council governs the organization's DEI&B deliverables and is driving long-term sustainable results, which is tracked with a dashboard to ensure continued progress. Among the deliverables is an approach to ensure DEI&B is a leadership core competency. In 2022 and 2023, all directors and above were required to participate in the Intercultural Development Index (IDI), an online assessment to benchmark a person's intercultural competency. Allina Health also requests a land acknowledgement announcement be read at the start of system meetings to honor indigenous people and their traditional, ancestral and contemporary lands.

Allina Health's DEI&B plan ensures the organization embeds DEI&B in its many roles as a provider, employer, community member, investor and purchaser. As an employer, Allina Health is committed to increasing and retaining employee diversity at all levels in the organization with a focus on leadership. Additional training on bias in hiring and bias in patient care is also provided to leaders throughout the organization to ensure that Allina Health's culture is welcoming, supportive and collaborative. This includes building infrastructure throughout the organization to create accountability for achieving progress. For example, Allina Health features seven Employee Resource Groups to provide opportunities to participate in courageous conversations around topics of inclusion and equity, to volunteer in the community and to attend culturally responsive trainings. DEI&B work creates the strategies, plan and systemwide approach to help bring to life Allina Health's commitment to providing and enhancing a workplace culture where all employees feel safe, respected and empowered to fulfill Allina Health's mission.

As a provider, Allina Health is dedicated to eliminating health disparities and variations in quality of care, while improving access and consumer experience for all care services. From culturally responsive care training to building capability among staff in patient-facing roles to implementing innovative care models to better meet patient needs,

Allina Health's health equity strategy strives to provide equitable, inclusive care for all. An example is offering culturally responsive personal care products in inpatient units across the system, including shampoos, conditioners, lotions, and disposable hijabs. Allina Health strives to reduce obstacles to health such as poverty, discrimination and their consequences.

The organization continues to advocate for investments that create innovative solutions to achieve enduring improvements in access, quality and health outcomes. This includes partnering with policymakers, employers and community advocates to remove barriers to health equity and support the physical and social recovery of South Minneapolis. Allina Health spent \$23 million in 2023 with diverse, local suppliers, with that number projected to grow in the coming years.

In 2023, Allina Health joined the Healthcare Anchor Network, a coalition of more than 75 health systems across the country working collaboratively to sustainably support long-term health and well-being of our communities by effectively collaborating with community stakeholders, implementing strategies around inclusive, local hiring and inclusive, local purchasing and more. Allina Health continually seeks opportunities to ensure all investments align with the organization's equity principles and values.

Impact Investments. With the approval of the Investment Committee of the Allina Health Board of Directors, Allina Health has created a separate portfolio and dedicated funds in the initial amount of \$30 million to make investments that create both positive social and environmental impact that is actively measured as well as positive financial return. The intent is to pursue the Allina Health organizational mission more effectively and use assets to generate impact toward improving the health of the communities it serves, while maintaining liquidity.

As part of this work, Allina Health has committed to providing impact investments totaling \$19.6 million as of March 31, 2024. Of the \$19.6 million committed, \$14.5 million has been closed to Minnesota Inclusive Growth Fund (MIGF), Local Initiatives Service Corporation (LISC), MSP Equity Fund, Tundra Venture Capital Fund, Mni Sota Fund, Fortis Capital, Neighborworks Home Partners, Greater MN Housing Fund, Capita3 Fund II, and Bread & Butter Fund IV. MIGF is designed to equip community small businesses and mission-driven small business lenders with the support required to meet the unique rebuilding and recovery needs of the communities they serve. LISC funds and supports projects to revitalize communities and bring great economic opportunity to the community. Investments will continue to be sourced and funded over the course of the next two-to-five years, with the potential for growth of the fund over time.

Sustainability efforts. As one of the region's largest health care providers, taking care of the environment is critical to ensure good health for Allina Health's patients, employees and community.

In 2020, Allina Health created a comprehensive sustainability program and in April 2021, Allina Health joined the Health Care Climate Challenge alongside more than 28,000 hospitals and health centers in 42 countries. In March 2023, Allina Health strengthened its commitment to care by formalizing a Climate Action Plan with the ambitious goals to reduce greenhouse gas emissions 50% by 2030 and reach net zero greenhouse gas emissions by 2050. With this plan, Allina Health also pledges to prepare for the impacts of extreme weather and to educate staff and communities on challenges and solutions related to climate and health.

At Allina Health, climate action includes both Allina Health's overall approach and to a collection of efforts and projects occurring across the system and at individual business units to create and maintain a healthy environment. The organization's energy and waste reduction efforts have saved Allina Health more than \$1 million annually.



The new Midwell Apartments, located near Abbott Northwestern Hospital in South Minneapolis, is one example of Allina Health's impact investments come to life. The 86-unit building provides affordable housing following Allina Health's impact investment with Greater MN Housing Fund.

Sustainability is also being considered through Allina Health's commitment to Leadership in Energy and Environmental Design (LEED) certification for major new construction projects. See "Other Recent Initiatives and Developments – Abbott Northwestern Hospital Infrastructure Updates" below.

Practice Greenhealth, a national organization for environmental sustainability within health care, has recognized Allina Health for organizational efforts to reduce greenhouse gas emissions, implementation of climate resilient infrastructures and advocating for healthy communities. In 2023, Practice Greenhealth recognized Allina Health with a System for Change Award and 13 Allina Health sites with Greenhealth Partner for Change Awards for commitment to environmental improvements.

Workforce Planning

People bring Allina Health's mission to life, which is why Allina Health is committed to investing in the organization's talent by fostering a relationship-driven culture rooted in its values. As one of Minnesota's largest employers, Allina Health recruits and retains exceptional people by offering valuable benefits that support the total well-being of its employees and their family members. Allina Health strives to empower and support employees so they can focus on delivering exceptional care. This further investment helps employees feel an even stronger connection to the organization and the work they do and help provide even better experiences and outcomes for patients.

At Allina Health, culture is a differentiator. Allina Health is mounting a robust talent acquisition campaign called [Careers with Purpose](#) to mitigate the national nursing shortage and local workforce challenges, highlighting current employees and their purpose through video. Through this campaign, Allina Health is telling stories of how people across a variety of disciplines are finding purposeful careers in the organization.

Workforce planning is an essential part of Allina Health's strategic plan to ensure patients receive the outstanding care they need for generations to come. Allina Health is engaging in numerous initiatives to support the organization's workforce retention and recruitment both in the near term as well as the long term. In the near term, Allina Health has established sub-initiatives dedicated to compensation management, talent acquisition optimization, improved retention and more efficient agency utilization.

Allina Health's long-term workforce planning initiatives focus on supporting the organization's existing and incoming talent with more resources and training opportunities to support their growth and development, including plans for an RN residency program, improved orientation programs, and development programs for nurse leadership. In August 2023, Allina Health and the University of Minnesota School of Nursing signed a formal academic-practice partnership, or collaboratory, to generate, disseminate, and apply knowledge for the improvement of nursing practice, education, and patient outcomes. The collaboratory fosters creativity and innovation while enhancing nursing education and professional development, research, practice, diversity, inclusion and equity, and recruiting top nursing talent. Allina Health continues to explore methods to identify workforce needs sooner and building and growing partnerships, including with educational institutions, to identify skilled, mission-driven individuals, as well as developing in-house expertise in coordinating staffing.

Allina Health's all together better mentality is an integral part of the organization's retention and culture-building efforts. Allina Health is proud to offer numerous support services and benefits to help employees access health care and well-being services. One example is through the Center for Provider Well-being, which is committed to promoting professional fulfillment through organizational and individual initiatives, research and education. The Center advances structures, processes, community and support so that every provider can flourish as a whole person. Through in-person and virtual resources, the Center offers coaching, counseling, peer support, group offerings, financial services, benefits support, quiet room, workstations and more. Allina Health also offers a variety of resources for all employees, including many new types of vendor



The Center for Provider-Well-being opened its physical space at Allina Health's headquarters in Minneapolis Wednesday, Jan. 17.

support for anxiety, sleep and financial support, and offered well-being funds for all employees to use on well-being products, memberships, services or gift cards of their choice.

Other Recent Initiatives and Developments

Investing In Abbott Northwestern Hospital, Our Quaternary Care Center

Allina Health's Board of Directors has approved approximately \$200 million of capital expenditures for infrastructure updates at Abbott Northwestern Hospital to maintain its status as the premier quaternary hospital in the Allina Health system. In March 2022, the Allina Health Board of Directors approved an additional \$1.2 billion to include a new building that will include surgical and critical care services and will be constructed in the heart of the campus. The 10-story, approximately 575,000 square foot building will replace the hospital's existing operating rooms and create new care environments for patients, visitors and staff. Allina Health is partnering with Mortenson as the General Contractor for the project, which began in late 2022. Allina Health anticipates funding a portion of the capital expenditures associated with the new surgical and critical care services building with a portion of the proceeds of the Series 2023 Bonds.

Richard M. Schulze and the Richard M. Schulze Family Foundation are helping to make this project possible with a \$25 million gift, the largest donation in the history of Allina Health.



Allina Health celebrated breaking ground on a new surgical and critical care services building in September 2023.

A new Transportation HUB, now called the Purple Ramp, accounts for approximately \$77 million of the initial \$200 million in infrastructure updates and serves Abbott Northwestern Hospital as well as Allina Health's headquarters and the Midtown Commons in South Minneapolis. The new structure, located at the southern end of the Abbott Northwestern Hospital campus, has been guided by Allina Health's commitment to sustainability and supports multi-modal transportation options. The Purple Parking Ramp supports bikes and other non-vehicle transportation, and it improves the look, feel and safety of the street level. This new facility enhances the safety and usability of Chicago Avenue, a main thoroughfare in South Minneapolis, and the sidewalk along the Greenway, a bike highway that runs adjacent to the campus. Improvements include new lighting and a pollinator garden with native, pollinator-friendly plantings along the Purple Ramp's 5,000-square-foot slope near the Midtown Greenway. The new pollinator habitat includes carefully selected plants based on their bloom time and ability to grow well and prevent erosion on a steep slope. Additionally, the roof of the structure will feature a community solar garden that is expected to come online in 2024.

Allina Health's Board of Directors has approved up to \$149 million of capital expenditures for a Central Utility Plant (CUP) project that will upgrade the hospital's utilities to 21st century standards, with more reliable, energy-efficient technology. This undertaking will improve the way the hospital heats and cools its buildings and sterilizes surgical instruments. The project has successfully received the necessary permits from the Minnesota Pollution Control Agency.

The MacMillan Education and Conference Center is a \$12 million addition to the Tenth Avenue Facility, made possible by the generosity of donors including Whitney and Elizabeth MacMillan. The site serves meeting, conference and educational needs for the entire Allina Health system, including a state-of-the-art simulation lab. The building is also available for use by community members and organizations.

Additional projects at Abbott Northwestern Hospital include an extensive remodel of the Piper Building to accommodate the Allina Health Cancer Institute, as well as build a new infusion center, compounding pharmacy and establish a link connection to Radiation Oncology and Oncology Clinic. See “Allina Health Cancer Institute” below.

Strategic Relationship with Optum

On Feb. 1, 2024, Allina Health and Optum announced a strategic relationship that enables Allina Health to leverage Optum’s innovative technology, resources and capabilities to build upon the organization’s existing Information Services and Revenue Cycle Management operations. This ensures Allina Health’s resources are prioritized toward direct care of the community and continued excellence in its administrative support to caregivers and patients. The relationship will allow Allina Health to:

- continue advancing information systems solutions to improve the patient and provider experience
- streamline the billing experience for patients and providers by implementing industry-leading technologies and processes across revenue cycle operations
- adopt innovative technologies at an accelerated pace.

Approximately 2,000 Allina Health employees in Information Services and Revenue Cycle Management became employees of Optum effective May 5, 2024, while maintaining their respective roles in supporting Allina Health’s patients and providers. Both organizations are committed to ensuring a smooth transition for these team members while maintaining seamless operations.

New Facilities in Cambridge

In December 2021, the Allina Health Board of Directors approved plans for state-of-the-art facilities to replace Cambridge Medical Center and redefine care in the community. In July of 2023, the project to build new facilities for Cambridge Medical Center was placed on hold as the organization navigates financial challenges. While there is no definitive timeline for the project right now, Allina Health remains committed to the Cambridge community and looks forward to creating and investing in the next generation of health care delivery services for the community when appropriate.

Center for Integrated Mental Health and Addiction Care

Allina Health is doubling down its commitment to mental health and addiction care with the Center for Integrated Mental Health and Addiction Care. The revolutionary facility, which opened to patients Feb. 29, 2024, transforms the delivery of mental health and addiction care by providing a comprehensive, connected and convenient location for patients and families, attached to Mercy Hospital – Unity Campus in Fridley. This highly integrated facility adds nearly 50,000 square feet of mental health and addiction care, with the full suite of Allina Health’s outpatient services as well as intensive day programs. The Center also brings community-centered programs together more closely to expand the capacity and reach of the organization’s collective mental health and addiction services, resulting in an enhanced model of providing safe, high-quality care for more patients in a specialized way.

The acquisition and renovation of the building that houses the Center for Integrated Mental Health and Addiction Care was funded by philanthropic giving led by the Allina Health Foundation.



The Center for Integrated Mental Health and Addiction Care is a highly integrated facility that is conveniently attached to Mercy Hospital, Unity Campus in Fridley.

Expanding Outpatient Services in the South Metro

Allina Health is transforming outpatient care options in the south metro with a new multi-specialty center and primary care clinic in the rapidly growing city of Lakeville. The new multi-specialty center brings the full Allina Health ambulatory platform to the south metro, including more than 20 specialties, such as orthopedics, oncology, women's health and cardiology, as well as an ambulatory surgery center. The specialty center opened to patients April 29, 2024.



Allina Health Lakeville North Clinic welcomed its first patients Oct. 16, 2023.

Additionally, Allina Health expanded services at Allina Health Apple Valley Clinic and opened Allina Health Savage Urgent Care in March 2022. The new primary care clinic, Allina Health Lakeville North Clinic, welcomed its first patients Oct. 16, 2023. In total, these investments will add more than 100,000 square feet of new health care real estate to the area and dramatically increase access to primary and complex specialty care.

Expanding Outpatient Services in Eden Prairie

Allina Health plans to expand outpatient services in Eden Prairie, including improved access to primary care and a multi-specialty clinic. This is an important investment in the growing west metro community that will dramatically expand access to our care offerings in the area.

This advancement will create market-leading access to primary care and specialty presence, including improved access to women's health, orthopedics and cardiovascular rehabilitation at our existing Allina Health – Eden Prairie Clinic supported by Allina Health Minneapolis Heart Institute. The expanded services are anticipated to be available by early 2025.

Allina Health Cancer Institute

Established in 2021, the Allina Health Cancer Institute (AHCI) has rapidly established itself as a regional destination for comprehensive, innovative and compassionate care. Medical, surgical and radiation oncologists span the care continuum and partner with specialists that include genetic counselors, nutritionists, financial navigators, mental health counselors and many more to provide each patient with care tailored to their unique needs and circumstances.

It's a level of care that has achieved recognition by national and local organizations and publications. Among AHCI's recent achievements:

- Abbott Northwestern Hospital was included in *Newsweek's* 2024 list of the World's Best Specialized Hospitals in oncology.
- Abbott Northwestern, Mercy and United hospitals are each rated as "high performing" in *U.S. News & World Report's* 2023-24 Best Hospitals rankings in categories that include colon cancer surgery, lung cancer surgery, leukemia, lymphoma and myeloma, and prostate cancer surgery.
- The AHCI Piper Breast Center – Minneapolis was recently accredited by the American College of Physicians' National Accreditation Program for Breast Centers – one of only four sites in Minnesota to have been accredited – for meeting the most rigorous standards in care.
- The surgical team at Abbott Northwestern Hospital received the Minnesota Hospital Association's "Innovation of the Year in Patient Care" award for completing a procedure that allows for the detection and removal of a lung cancer mass under a single dose of anesthesia.

AHCI is committed to research, knowing that today's clinical trials are tomorrow's life-changing treatments. The Institute is on the forefront of medical innovation, currently engaging in numerous clinical research trials, including a first-of-its-kind trial that uses artificial intelligence and a blood draw to revolutionize cancer diagnosis and treatment.

AHCI has three nationally accredited comprehensive breast centers at Abbott Northwestern, Mercy and United hospitals. The Piper Breast Center Mercy Hospital is the first of its kind in the north metro and includes state-of-the-art screening, diagnostics and treatment for breast cancer, as well as breast cancer education, genetic counseling and high-risk breast cancer programs and includes the radiologist, oncology surgeon and medical oncologists all in one setting. Allina Health's multidisciplinary breast centers are also located at its other comprehensive cancer sites at Abbott Northwestern and United hospitals, and treatment is available at infusion centers located throughout the community.

Multidisciplinary, integrated care is also a pillar of AHCI's approach to both colorectal cancer care – where surgical, medical and radiation oncologists work closely with advanced practice providers, nurses, nurse navigators, cancer researchers and others to determine the most effective course of treatment – and skin cancer care, which brings together oncologists, dermatologists, social workers, cancer rehab specialists and others to serve patients.

Allina Health Ambulatory Surgery Center Strategy

Allina Health is committed to expanding its ambulatory surgery platform to enable efficient, cost-effective and high-quality consumer-facing care consistent with its payer strategies and intended to keep pace with rapidly evolving reimbursement models. The strategy, which involves commitment of capital and other resources, is aligned to advance Allina Health's vision as the high-value, integrated complex care coordinator for its community. This propels Allina Health as an organization that can deliver surgical care in all settings from complex surgery in a major metropolitan hospital to same day surgery in a community-based setting.

Since 2020, Allina Health has partnered with SCA Health to jointly own ambulatory surgery centers across the Allina Health service area with SCA Health running the centers' operations and strategically partnering to enable Allina Health to become the market share leader in surgery center space in the Twin Cities. SCA Health is a nationally renowned group with a network of more than 250 ambulatory surgery centers performing more than one million procedures per year. WestHealth Surgery Center, one of Allina Health's first ambulatory surgery centers in partnership with SCA Health, was named the top ambulatory surgery center in the Twin Cities by Newsweek (#2 in Minnesota) in 2023.

As of March 2024, Allina Health's ambulatory surgery center portfolio features nine locations, including a first-to-market cardiovascular surgery center in Edina which opened in September 2022. The newest ambulatory surgery center within the Lakeville specialty center opened in February 2024.

Allina Health Neuroscience, Spine and Pain Institute

Allina Health Neuroscience, Spine and Pain Institute (AHNSPI) has quickly become a regional leader in this complex care, with more than 109,000 hospital visits for neurological and spine care in 2023. AHNSPI aligns, coordinates and builds upon the extensive talent and clinical programs across the system to create a patient-focused approach to care that supports a patient from disease prevention to quaternary care and post-acute to home care.

The Institute's level of expertise and quality of care has been recognized on a regional and global level:

- Abbott Northwestern Hospital was included in *Newsweek's* 2024 list of the World's Best Specialized Hospitals in neurosurgery and neurology.
- Abbot Northwestern Hospital was rated as "high performing" by *U.S. News & World Report's* 2023-24 Best Hospitals rankings for neurology & neurosurgery, back surgery, and stroke.
- Mercy and United hospitals were similarly recognized as high performing" by *U.S. News & World Report* in the category of stroke.

AHNSPI provides patients with access to a range of surgical and medical spine therapies – as well as interventional pain services – both at Allina Health’s metro area hospitals as well as at its regional medical centers. Stroke patients and their families can access advanced care at Abbott Northwestern Hospital and United Hospital—both certified as Comprehensive Stroke Centers by DNV Healthcare—as well as at Mercy Hospital, a certified Primary Plus Stroke Center, and Mercy Hospital – Unity Campus, a certified Primary Stroke Center. Mercy Hospital is also an accredited Thrombectomy-Capable Stroke Center. Patients outside the Twin Cities metro area can also rely on top-of-the-line care at Minnesota Department of Health-certified acute stroke ready facilities at Buffalo Hospital, Cambridge Medical Center, New Ulm Medical Center, Owatonna Hospital, Regina Hospital – Hastings, River Falls Area Hospital, and St. Francis Regional Medical Center. Through its telestroke program, offered at more than 30 hospitals throughout Minnesota, South Dakota and Wisconsin, AHNSPI is also able to connect patients and providers to an Allina Health neurologist at any time of day.

AHNSPI also features a Level 4 Epilepsy Center at Abbott Northwestern Hospital and neurocritical care units at Abbott Northwestern and United hospitals.

2023 and early 2024 Labor Negotiations

The results of the NLRB election petition filed by Doctors Council SEIU Local 10MD covering physicians at Mercy Hospital were finalized on January 26, 2024. The parties are preparing to negotiate a first contract and we expect negotiations to begin in June 2024. Negotiations for a first contract covering primary and urgent care providers began in February 2024. Nine bargaining schedules have been held. We expect that the negotiations for each contract will continue through at least the end of 2024.

Negotiations for a successor contract covering the skilled maintenance employees at the metro-area hospitals are under way. The contract expires on April 30, 2024.

Change Healthcare cyberattack

Change Healthcare (Change), a division of UnitedHealth Group (UHG), was the subject of a cyberattack in late February 2024. Change provides electronic prescribing, electronic payment capability, insurance batch eligibility, and electronic medical claims (billing) services to the System. As a result of the cyber event, Allina Health disconnected from Change, resulting in an inability to bill certain claims, verify insurance coverage and verify preauthorization status for hospital services. Allina Health also activated manual workarounds for affected areas.

Due to the inability to bill certain claims, Allina Health drew \$200,000,000 on an existing line of credit for liquidity purposes. On April 11, 2024, Allina Health paid back the full amount of the \$200,000,000 draw.

The duration and extent of this business interruption are being actively monitored by management. As of March 5, 2024, electronic prescribing had securely returned to regular operations. In addition, Allina Health has transitioned a majority of the billing and insurance services back to Change. The System is actively working with its commercial payers to ensure patient billings are filed as timely and as accurately as possible.

Updates to Credit Ratings Participation

In August 2023, Allina Health announced it ended its engagement with Moody’s Investor Services (Moody’s) for the rating of Allina Health bonds. The decision to end Moody’s engagement was based on Allina Health’s initiatives to reduce expenses and a determination that three ratings were not required or providing significant value to the organization. The decision was not made to prevent or cure a default or event of default (or reduce any applicable spread) under any debt agreement to which Allina Health is a party.

Allina Health expects to maintain two ratings and continues to engage Standard & Poor’s Global Ratings (“S&P”) and Fitch Ratings (“Fitch”) to provide ratings with respect to Allina Obligated Group debt.

Allina Health has been informed that Moody's intends to continue to rate such debt for some period of time without any expected future communication from or information provided directly by Allina Health. Allina Health does not know how long Moody's will continue to rate Allina Health debt.

Credit Ratings

In April 2024, Fitch Ratings changed Allina Health's bond rating to A+. The outlook is stable.

In February 2024, Standard & Poor's Ratings Services changed the rating of Allina Health to A+. The outlook is stable.

The complete rating agency reports are available at dacbond.com; fitchratings.com; or standardandpoors.com.

Management Personnel

Following are the Executive Leadership Team members and their biographies (as of May 5, 2024):



Lisa Shannon (58) – *President and Chief Executive Officer*. In her role, Lisa Shannon helps drive exceptional clinical, operational and financial performance, so that Allina Health can deliver quality, affordable care to the communities it serves. The Allina Health Board of Directors appointed Ms. Shannon as the successor to Dr. Penny Wheeler as Chief Executive Officer effective January 1, 2022. Prior to Allina Health, Ms. Shannon served as president, Health System Delivery, for KentuckyOne Health, Kentucky's largest integrated health system. Before joining KentuckyOne Health, Ms. Shannon served in several executive leadership roles at Spectrum Health in Grand Rapids, Michigan, including three years as Chief Operating Officer. Ms. Shannon began her career as a clinical dietitian at The Community Hospital in Springfield, Ohio, and spent 10 years at OhioHealth Corporation in Columbus, rising to vice president of ambulatory services. She is a graduate of Bowling Green State University in Ohio and earned a Master's of Business Administration from Franklin University in Columbus, Ohio. Ms. Shannon places a great deal of value on community engagement. She currently serves on the Board of the Greater Twin Cities United Way and the University of St. Thomas Opus College of Business Strategic Board of Advisors.



Richard Magnuson (61) – *Executive Vice President, Chief Financial Officer*. Mr. Magnuson was appointed Chief Financial Officer in January 2017. He is responsible for ensuring that Allina Health resources are deployed properly to enhance the quality, safety and effectiveness of the care Allina Health provides to its patients and communities. Prior to joining Allina Health, Mr. Magnuson was the Chief Financial Officer of City of Hope, based in California. Mr. Magnuson previously held various finance and executive positions with Group Health in Seattle, Washington, and Fletcher Allen Health Care in Burlington, Vermont. Mr. Magnuson also served Allina Health from 1987 to 2004 in a variety of financial leadership roles. In addition to Finance, Mr. Magnuson also has operational responsibility for Payer Relations and Contracting, Supply Chain Management and Revenue Cycle Management. Mr. Magnuson serves as treasurer on the board of directors for Portico Healthnet, an organization dedicated to making health care accessible and affordable for all Minnesotans. In 2019, Mr. Magnuson was recognized by Minneapolis/St. Paul Business Journal as CFO of the Year for his work in helping streamline and improve the financial position of Allina Health. He earned his bachelor's degree at Augsburg College in Minneapolis in accounting and finance and his master's degree at the University of St. Thomas in Minneapolis.

In January 2024, Mr. Magnuson announced his departure from Allina Health effective in early April after 25 years of service to the organization. Doug Watson joined the organization in January 2024 as interim finance executive while the organization conducts a national search for a chief financial officer. Mr. Watson previously served as Interim Executive Vice President and System Chief Financial Officer for Sharp Healthcare and has executive experience at UnityPoint Health, CommonSpirit and PeaceHealth.



Elizabeth Truesdell Smith (61) – *Senior Vice President, General Counsel and Secretary to the Allina Health Board of Directors.* Elizabeth Truesdell Smith has been practicing law for than more than two decades and has served as in-house counsel for Allina Health since December 2000. As general counsel, Ms. Truesdell Smith serves as the chief legal advisor to the health system and all of its operating units, subsidiaries, employees, boards and constituents. She also is the senior executive responsible for system clinical risk management and all insurance programs. Prior to her appointment as general counsel, she held the position of vice president of the Allina Health Law Department medical-legal team. Before joining Allina Health, Ms. Truesdell Smith practiced as a trial lawyer for eight years with a focus on hospital and health law, medical professional liability and medical products liability litigation defense. Ms. Truesdell Smith received her juris doctor from the University of Minnesota Law School in 1992 after earning a master's degree in public health with a health law and bioethics concentration from Boston University School of Public Health in 1989.



Christine Moore (54) – *Executive Vice President, Chief Administrative Officer.* Christine Moore leads the Human Resources, Philanthropy, DEI&B, Marketing, Communications, Public Affairs and Community Benefit and Engagement, Compliance and Security functions of Allina Health and is accountable for the organization's human resources and people strategies, which are critical to building and sustaining an engaged workforce committed to exceptional care and service. Prior to joining Allina Health in 2015, Ms. Moore served as an executive HR leader at Ecolab and Best Buy. She began her career at a management consulting agency. Ms. Moore earned a bachelor's degree in economics from Scripps College in Claremont, California, and she received her PhD in Organizational Psychology from Claremont Graduate University. She is on the board of the Montessori Center of Minnesota and the board of St. Catherine University



Lawrence Cho (46) – *Senior Vice President, Chief Strategy and Growth Officer.* Lawrence Cho is responsible for advancing and sustaining Allina Health's innovation, strategic and growth initiatives, while prioritizing accelerating organizational performance. Before joining Allina Health in 2021, Mr. Cho was a member of Mayo Clinic's executive team. At Mayo Clinic, he held positions of chief operating officer for Mayo Clinic International, where he focused on expanding Mayo's model of care internationally, and chair of Corporate Development, where he led enterprise-wide strategic partnership and merger & acquisitions initiatives. Prior to joining Mayo Clinic, Mr. Cho held various finance, strategy and business development positions in industry-leading organizations of Medtronic, McKinsey & Company and General Mills. Mr. Cho earned both a bachelor's degree in neuroscience and economics and a Master of Business Administration from the University of Michigan. Past board memberships include the Make-A-Wish Foundation, Wolverine Venture Fund and Mu Performing Arts.



Hsieng Su, MD (56) – *Senior Vice President, Chief Medical Executive.* Dr. Hsieng Su serves as the senior executive responsible for clinical performance and advancing Allina Health's safety, quality and population health journey. Prior to joining Allina Health in 2021, Dr. Su was at SSM Health-St. Louis as the Regional Chief Medical Officer, where she was an integral part of improving patient care by leading clinical teams across five specialty areas, creating a physician leadership development program, providing leadership to the SSM Health-St. Louis Institutional Review Board and research contracts with the affiliated academic university. She served as a board member of the Accountable Care Organization for the region. Dr. Su spent six years at Aurora St. Luke's Medical Center in Milwaukee, Wisconsin, as the Chief Medical Officer prior to her time with SSM Health-St. Louis. Dr. Su holds an undergraduate degree in microbiology from the University of Washington, Seattle; a Master of Business Administration from the University of California, Irvine; earned a Master of Health Care Management from Harvard School of Public Health; and a doctorate of medicine from Georgetown University School of Medicine. She completed her internal medicine residency at Mayo Clinic, finished a preventative medicine fellowship at Mayo Clinic, and completed The Academy, GE Chief Medical Officer fellowship.



D'Andre Carpenter, DNP, RN (41) – *Senior Vice President, Chief Nursing Executive*. Dr. Carpenter is responsible for the development and implementation of strategies that improve patient satisfaction, operational excellence and clinical service delivery for the organization. He leads nursing workforce development and programs that promote the retention, recruitment, continuing education and development of clinical nurses and support services staff. Prior to joining Allina Health in 2023, Dr. Carpenter was senior vice president, system chief nursing officer at UnityPoint Health in Iowa, where he led system nursing practice and clinical care and led a nursing strategic plan that promoted a responsive, flexible, nursing workforce. Dr. Carpenter has held executive nursing leadership, clinical informatics and data analytics positions at Jefferson Health in Philadelphia, Baylor Scott & White health in Dallas and VCU Health System in Richmond, VA. Dr. Carpenter earned undergraduate degrees in Computer Science and Nursing; an Executive Master of Science in Information Systems Management and Master of Science in Nursing from Virginia Commonwealth University; and a Doctor of Nursing Practice degree from Old Dominion University. He currently serves on the Academy Fellowship Program Advisory Board for the Healthcare Management Academy in Arlington, VA, the Talent Innovation Advisory Board for AMN Healthcare in Dallas, and the Nursing Executive Advisory Board for AvaSure Health in Belmont, MI.



Dominica Tallarico (57) – *Executive Vice President, Chief Operating Officer*. Dominica Tallarico started at Allina Health in the new role of Executive Vice President and Chief Operations Officer on May 1, 2023. Dominica is responsible for implementing Allina Health's strategic plans into daily operations to meet the organization's objectives. Dominica most recently served as the executive vice president of the Illinois Market for the newly formed Advocate Health (previously Advocate Aurora Health). She was the executive leader responsible for acute and ambulatory strategy and operations in Illinois representing nine hospitals, a children's hospital, 400 ambulatory sites of care and medical group clinics, 6,300 physicians of which nearly 1,800 are employed, and \$7 billion in revenues. During her tenure, she helped to strategically grow the organization's acute and ambulatory network and sites of care, including population health capabilities and clinical service lines, serving two million people in Illinois. She is a strategic thought leader, cultivating innovation as evidenced through establishing innovative care models and partnerships including One Medical, Surgical Care Affiliates, MD Anderson affiliation, and digital consumer-facing products. Dominica holds a bachelor's degree from Loyola University Chicago and a master's degree in health administration from Rush University, and is a board member for Second Harvest Heartland.

System Governance

Allina Health is governed by a single Board of Directors ("Board") that meets on a quarterly basis over a two-day period, including Committee meetings. The board oversees the operations of the entire Allina Health system. Allina Health's regional hospitals have advisory councils, which function in an advisory capacity. Allina Health subsidiaries have separate boards which are subject to the reserved powers of the Allina Health Board.

In addition to the directors elected by the Board, the Chief Executive Officer of Allina Health serves ex-officio, with the right to vote. The bylaws prescribe three-year terms for elected directors and limit elected directors to three (3) successive terms. Upon a recommendation by the Governance & Nominating Committee, an elected director may be elected to serve for additional terms of one (1) to three (3) years.

The Board retains approval authority over all significant strategic and policy decisions. Many decisions are approved by the Board through its approval of the consolidated annual strategic and financial plan. Capital expenditures over a certain level and any significant debt incurrence require separate action by the Board.

The Allina Health Board has eight standing committees: Audit & Compliance; Human Resources & Compensation; Finance; Investment; Governance & Nominating; Philanthropy; Physician Governance; and Quality & Population Health.

Awards and Recognition

Allina Health is honored to be the recipient of numerous awards and certifications that recognize the organization's commitment to delivering safe, high-quality care, including:

- **U.S. News & World Report Best Hospitals** named Abbott Northwestern (#1), Mercy (#2, tie) and United (#5, tie) hospitals best in Twin Cities and Minnesota. Buffalo Hospital was recognized for the third consecutive year. River Falls Area Hospital also earned high performing ranking in pneumonia.
- Abbott Northwestern, Mercy and United hospital and St. Francis Regional Medical Center were recognized by the American Heart Association's Get With The Guidelines®– Stroke program. This is an in-hospital program for improving stroke care by promoting consistent adherence to the latest scientific treatment guidelines.
- The **2023 Practice Greenhealth Environmental Excellence Awards** recognized 13 Allina Health sites with their Partner for Change Award: Abbott Northwestern Hospital, Buffalo Hospital, Cambridge Medical Center, Courage Kenny Rehabilitation Institute in Golden Valley, Faribault Medical Center, Mercy Hospital, Mercy Hospital – Unity Campus, New Ulm Medical Center, Owatonna Hospital, River Falls Area Hospital, St. Francis Regional Medical Center, United Hospital and United Hospital – Hastings Regina Campus.

Visit allinahealth.org/about-us/awards-and-recognition to see more.

Allina Health System

Consolidated Balance Sheets

(Unaudited)
(Dollars in thousands)

Assets	March 31, 2024	December 31, 2023
Current assets:		
Cash and cash equivalents	\$ 390,127	227,248
Short-term investments	661,172	793,399
Patient accounts receivable	684,748	580,331
Inventories and supplies	106,502	101,741
Other current assets	276,497	227,021
Total current assets	2,119,046	1,929,740
Investments	1,708,178	1,667,312
Investments with limited uses	535,336	556,530
Operating lease right-of-use assets, net	211,980	216,413
Land, buildings, and equipment, net	1,514,835	1,510,090
Other assets	305,099	302,915
Total assets	\$ 6,394,474	6,183,000
Liabilities and Net Assets		
Current liabilities:		
Accounts payable and accrued expenses	\$ 536,427	583,753
Current portion of long-term debt	22,321	23,200
Current portion of operating lease liabilities	35,506	35,202
Revolving line of credit	380,000	180,000
Other current liabilities	221,644	172,844
Total current liabilities	1,195,898	994,999
Long-term debt	1,682,044	1,684,466
Operating lease liabilities	202,907	207,405
Other liabilities	198,803	194,710
Total liabilities	3,279,652	3,081,580
Net assets:		
Without donor restriction		
Controlling interest	2,867,580	2,868,724
Noncontrolling interest in subsidiaries	25,979	25,896
Total net assets without donor restrictions	2,893,559	2,894,620
With donor restriction	221,263	206,800
Total net assets	3,114,822	3,101,420
Total liabilities and net assets	\$ 6,394,474	6,183,000

See accompanying notes to consolidated financial statements.

Allina Health System

Consolidated Statements of Operations and Changes in Net Assets

(Unaudited)
(Dollars in thousands)

	Three Months Ended	
	March 31, 2024	March 31, 2023
Revenues:		
Patient service revenue	\$ 1,234,103	1,155,300
Other operating revenue	113,383	91,101
Total revenues	<u>1,347,486</u>	<u>1,246,401</u>
Expenses:		
Salaries and benefits	852,070	851,153
Supplies and services	339,695	324,338
Depreciation and amortization	51,261	52,237
Financing costs	15,556	16,454
State assessments and taxes	25,524	23,054
Utilities and maintenance	20,450	24,565
Other operating expenses	62,403	56,181
Total expenses	<u>1,366,959</u>	<u>1,347,982</u>
Operating income (loss) before restructuring expenses	(19,473)	(101,581)
Restructuring expenses	<u>28,866</u>	<u>—</u>
Operating loss	(48,339)	(101,581)
Nonoperating gains (losses):		
Investment return	48,220	84,099
Gain (loss) on interest rate swap agreements	—	(4,314)
Other	(983)	(929)
Excess (deficiency) of revenues over expenses	(1,102)	(22,725)
Less (income) loss attributable to noncontrolling interests	<u>(679)</u>	<u>(531)</u>
Excess (deficiency) of revenues over expenses - attributable to controlling interests	<u>\$ (1,781)</u>	<u>(23,256)</u>

Allina Health System

Consolidated Statements of Operations and Changes in Net Assets

(Unaudited)
(Dollars in thousands)

	Three Months Ended	
	March 31, 2024	March 31, 2023
Net assets without donor restrictions, controlling interest:		
Excess (deficiency) of revenues over expenses \$	(1,781)	(23,256)
Net assets released from restrictions for capital purposes	652	10,622
Other	(15)	63
(Decrease) increase in net assets without donor restrictions	(1,144)	(12,571)
Net assets without donor restrictions, noncontrolling interest:		
Excess (deficiency) of revenues over expenses	679	531
Distributions to noncontrolling interests	(1,031)	(1,018)
Other	435	(14)
Increase (decrease) in net assets without donor restrictions, noncontrolling interest	83	(501)
Net assets with donor restrictions:		
Contributions	7,717	1,096
Investment return	8,448	5,285
Net assets released from restrictions	(1,966)	(11,498)
Other	264	213
Increase (decrease) in net assets with donor restrictions	14,463	(4,904)
Increase (decrease) in net assets	13,402	(17,976)
Net assets at beginning of year	3,101,420	3,226,125
Net assets at end of quarter	\$ 3,114,822	3,208,149

See accompanying notes to consolidated financial statements.

Allina Health System

Consolidated Statement of Cash Flows

(Unaudited)
(Dollars in thousands)

	Three Months Ended	
	March 31, 2024	March 31, 2023
Cash flows from operating activities:		
Increase (decrease) in net assets	\$ 13,402	(17,976)
Adjustments to reconcile decrease in net assets to net cash and cash equivalents used in operating activities:		
Depreciation and amortization	51,261	52,237
Gain on sale of land, buildings, and equipment	202	(1,923)
Change in fair value of interest rate swaps	—	3,499
Change in realized and unrealized (gains) loss on investments, net	(31,449)	(71,863)
Restricted contributions including cash for long-lived assets	(7,717)	(1,096)
Earnings on unconsolidated entities	(5,248)	(3,241)
Distribution to noncontrolling interest	1,031	1,018
Distributions received from unconsolidated entities	8	8
Changes in assets and liabilities:		
Contracted Payer Advances	109,460	—
Patient accounts receivable and other current assets	(159,105)	(40,984)
Accounts payable and accrued expenses and other current liabilities	(107,986)	(176,770)
Other assets and liabilities	5,156	2,326
Net cash and cash equivalents used in operating activities	(130,985)	(254,765)
Cash flows from investing activities:		
Proceeds from sales of land, buildings, and equipment	771	22
Purchases of land, buildings, and equipment	(56,979)	(47,663)
Contributions of cash for long-lived assets	—	110
Sales (purchases) of investments, net	144,004	199,898
Net cash and cash equivalents provided by investing activities	87,796	152,367
Cash flows from financing activities:		
Restricted contributions, net	8,168	1,650
Distributions to noncontrolling interests	(1,031)	(1,018)
Proceeds from revolving lines of credit draws	200,000	—
Refinancing and principal payments of long-term debt	(1,069)	(16)
Net cash and cash equivalents provided by financing activities	206,068	616
Increase (decrease) in cash and cash equivalents	162,879	(101,782)
Cash and cash equivalents at beginning of year	227,248	237,047
Cash and cash equivalents at end of quarter	\$ 390,127	135,265

See accompanying notes to consolidated financial statements.

Allina Health System

Abridged Notes to the Consolidated Financial Statements

Three Months Ended March 31, 2024 and 2023

(Unaudited)

(Dollars in thousands)

1. Patient Revenue and Accounts Receivable

Patient service revenue is reported at the amount that reflects the consideration to which Allina Health expects to be entitled in exchange for providing patient care. These amounts are due from patients, third-party payers (managed care and government programs), and others and includes variable consideration for retroactive revenue adjustments due to settlement of audits, reviews, and investigations. Revenue is recognized as performance obligations are satisfied.

Performance obligations are determined based on the nature of the services provided. Revenue for performance obligations satisfied over time is recognized based on actual charges incurred in relation to total charges. Performance obligations satisfied over time relate to inpatient acute care and outpatient services. Revenue for performance obligations satisfied at a point in time is recognized when goods or services are provided and Allina Health does not believe it is required to provide additional goods or services to the patient. Performance obligations for patients that are in house at period end generally complete within days or possibly weeks of period end, therefore are estimated on a pro rata basis.

Allina Health determines the transaction price based on standard charges for goods and services provided, reduced by contractual adjustments provided to third-party payers, discounts provided to uninsured patients in accordance with Allina Health policy, and /or implicit price concessions provided to uninsured patients. Estimates of contractual adjustments and discounts, or explicit price concessions, are based on contractual agreements, discount policy (or policies), and historical experience. The estimate of implicit price concessions is based on historical collection experience.

Agreements with third-party payers typically provide for payments at amounts less than established charges. Payment arrangements with major third-party payers included Medicare, Medicaid and managed care payers.

Laws and regulations concerning government programs, including Medicare and Medicaid, are complex and subject to interpretation. Compliance with such laws and regulations may also be subject to future government review and interpretation as well as significant regulatory action, including fines, penalties, and potential exclusion from the related programs. There can be no assurance that regulatory authorities will not challenge Allina Health's compliance with these laws and regulations, and it is not possible to determine the impact (if any) such claims or penalties would have upon Allina Health. In addition, contracts with commercial payers also provide for retroactive audit and review of claims.

Settlements with third-party payers for retroactive adjustments due to audits, reviews, or investigations are considered variable consideration and are included in the determination of the estimated transaction price for providing patient care.

These settlements are estimated based on the terms of the payment agreement with the payer, correspondence from the payer and historical settlement activity, including an assessment to ensure that it is probable that a significant reversal in the amount of cumulative revenue recognized will not occur when the uncertainty associated with the retroactive adjustment is subsequently resolved. Estimated settlements are adjusted in future periods as adjustments become known (i.e., new information becomes available), or as years are settled or are no longer subject to such audits, reviews, and investigations. Allina Health utilizes a process to identify and appeal certain settlements by government payers. Additional reimbursement is recorded in the year the appeal is successful.

Generally patients who are covered by third-party payers are responsible for related deductibles and coinsurance, which vary in amount. Allina Health also provides services to uninsured patients, and offers those uninsured patients a discount, either by policy or law, from standard charges. Allina Health estimates the transaction price for patients with deductibles and coinsurance and from those who are uninsured based on historical experience and current market conditions. The initial estimate of the transaction price is determined by reducing the standard charge by any contractual adjustments, discounts, and implicit price concessions. Subsequent changes to the estimate of the transaction price are generally recorded as adjustments to patient service revenue in the period of the change.

Allina Health provides care to patients regardless of their ability to pay. Therefore, Allina Health has determined it has provided implicit price concessions to uninsured patients and patients with other uninsured balances (e.g., co-pays and deductibles). The implicit price concessions included in estimating the transaction price represent the difference between amounts billed to patients and the amounts Allina Health expects to collect based on its collection history with those patients.

Patients who meet Allina Health's criteria for charity care are provided care without charge or at amounts less than established rates. Such amounts determined to qualify as charity care are not reported as revenue.

2. Fair Value Disclosures

Allina Health determines the fair value of its financial instruments based on the fair value hierarchy established in the Accounting Standards Codification (ASC) Topic 820, Fair Value Measurements, which requires an entity to maximize the use of observable inputs and minimize the use of unobservable inputs when measuring fair value. The standard describes three levels of inputs that may be used to measure fair value.

Level 1 Inputs: Quoted prices in active markets for identical assets or liabilities that the reporting entity has the ability to access at the measurement date.

Level 2 Inputs: Inputs other than quoted prices included within Level 1 that are observable for the asset or liability, either directly or indirectly through corroboration with observable market data.

Level 3 Inputs: Unobservable inputs for the asset or liability, that is, inputs that reflect the reporting entity's own assumptions about the assumptions market participants would use in pricing an asset or liability (including risk assumptions) developed based on the best information available in the circumstances.

Inputs and valuation techniques for significant other observable and significant unobservable inputs are:

For level 2 cash equivalents and fixed income assets that rely on significant other observable inputs and significant unobservable inputs, Allina Health employs multiple third-party information providers to help determine the fair value of the assets. Level 2 securities in separately managed accounts are held at Bank of New York Mellon ("BNYMellon"), who acts as Trustee and Custodian for the assets. As Custodian, BNYMellon uses multiple pricing services to value the assets. The investment managers utilize their own pricing services and valuation processes. Any significant discrepancies between Custodian and investment manager values are reconciled on a monthly basis by the managers and BNYMellon. Allina Health also employs an investment consultant who researches significant pricing differences between the manager and custodian. The consultant will notify the Custodian of any significant pricing issues.

For limited partnership assets and commingled monthly valued funds, Allina Health utilizes net asset value per share or its equivalent to determine the fair value of the assets. Allina Health has the ability to redeem its investment with the investee at net asset value per share (or its equivalent) at the measurement date, subject to notification period and other requirements specific to each investee.

Allina Health's financial assets and liabilities that are measured at fair value on a recurring basis were recorded using the fair value hierarchy at March 31, 2024, as displayed in the table below. Certain investments that are measured at fair value using the net asset value per share (or its equivalent) as a practical expedient have not been classified in the fair value hierarchy. The fair value amounts presented in the table are intended to permit reconciliation of the fair value hierarchy to amounts presented in the statements of consolidated balance sheets.

	Total	Fair Value Measurements Using		
		Level 1	Level 2	Level 3
Cash and Cash Equivalents				
Cash	\$ 101,777	101,777	-	-
Money Market Funds	288,350	288,350	-	-
Total Cash and Cash Equivalents	\$ 390,127	390,127	-	-
Investments - Trading Securities				
Short-Term Fixed and Money Market	\$ 333,821	333,132	689	-
Equity	380,043	135,340	244,703	-
Fixed Income	1,287,484	458,246	829,238	-
Investments Accounted for at Net Asset Value	356,345			
Total Investments - Trading Securities	\$ 2,357,693	926,718	1,074,630	-
Total Direct and Impact Investments	\$ 11,657			
Total Unrestricted Cash and Investments	\$ 2,759,477	1,316,845	1,074,630	-
	Total	Fair Value Measurements Using		
		Level 1	Level 2	Level 3
Investments with Limited Uses				
Short-Term Fixed Income	\$ 13,257	13,212	44	-
Money Market Fund	349,736	349,736	-	-
Equity	24,794	8,830	15,965	-
Fixed Income	75,236	27,945	47,291	-
Restricted Foundation Trusts	3,182	-	3,182	-
Investments Accounted for at Net Asset Value	69,131			
Total Investments with Limited Uses	\$ 535,336	399,723	66,482	-
Total Cash and Investments	\$ 3,294,813	1,716,568	1,141,112	-

Fair value measurements of investments in certain entities that calculate net asset value per share (or its equivalent) as of March 31, 2024, is as follows:

	Net Asset Value*	Unfunded Commitments	Redemption Frequency (if currently eligible)	Redemption Notice Period	Redemption Settlement Period
Private Capital Funds	\$ 425,476	\$ 205,407	Not redeemable	NA	NA

*Includes restricted and unrestricted assets

Private Capital Funds represent investments in private equity, private credit, and real assets. These funds include diversified approach and include limited partnership investments in the following categories: health care services and information technology companies, venture capital investments in health care companies, early growth stage health care private equity investments, senior secured and floating rate loans to middle market U.S. companies, co-investments in intermediate-term opportunities sourced by absolute return, private capital, and real asset managers, value-add and opportunistic real estate, special situation and other opportunistic investments, distressed and opportunistic real estate investments, single family rental properties, investments in the financial services industry, investments in companies in the energy industry, and buyout investments globally. The value of the investments in this category has been estimated using the net asset value per share of the investments.

3. Derivative Instruments

Allina Health used interest rate swaps as a part of its risk management strategy to manage exposure to fluctuations in interest rates and to manage the overall cost of its debt. Three of the four interest rate swaps were used to hedge identified debt and interest rate exposures and were not used for speculative purposes. One of the interest rate swaps was issued in advance of an expected debt issuance and was not tied directly to any outstanding debt. Interest rate swaps were recognized as either other long-term assets or other long-term liabilities in accordance with the netting provisions in the counterparty agreement and were measured at fair value.

Allina Health accounted for its interest rate swaps as required by accounting guidance for derivatives and hedging. Allina Health's interest rate swaps were not designated as effective hedges for accounting purposes. Gains or losses resulting from changes in the fair values of the interest rate swaps are reported as non-operating gains or losses. Any differences between interest received and paid under swap agreements are reported with the change in fair value of the swaps as non-operating gains or losses.

Effective June 29, 2023, Allina Health terminated all four interest rate swaps, for a total termination amount of \$30,184.

The following table provides details regarding the gains and (losses) from Allina Health's derivative instruments in the consolidated statements of operations, none of which were designated as hedging instruments.

	Amount of gain (loss) on change in fair value recognized as non-operating: interest rate swap agreements		Amount of interest paid to counterparty recognized as non-operating: interest rate swap agreements		Totals	
	Three Months ended Mar, 31 2024	2023	Three Months ended Mar, 31 2024	2023	Three Months ended Mar, 31 2024	2023
2009 B&C	\$ -	\$ (2,084)	\$ -	\$ (350)	\$ -	\$ (2,434)
2007C	-	(912)	-	(210)	-	(1,122)
2001	-	(503)	-	(255)	-	(758)
	\$ -	\$ (3,499)	\$ -	\$ (815)	\$ -	\$ (4,314)

4. Contingencies and Litigation

Laws and regulations governing the Medicare program are complex and subject to interpretation. As a result, there is at least a reasonable possibility that recorded estimates will change by a material amount in the near term. Allina Health believes that it is in compliance with all applicable laws and regulations and is not aware of any pending or threatened investigations involving allegations of potential wrongdoing that would have a material effect on its consolidated financial statements. Compliance with such laws and regulations can be subject to future government review and interpretation, as well as significant regulatory action, including fines, penalties, and exclusion from the Medicare program.

Allina Health is subject to various legal proceedings and claims that are incidental to its normal business activities. With respect to these actions, established reserves are fairly stated, though actual results could vary from the estimates and assumptions that were used.

5. Subsequent Events

Allina Health has evaluated subsequent events from the consolidated balance sheet date through May 15, 2024, the date at which the consolidated financial statements were distributed, and determined there are no other items to disclose.

Allina Obligated Group

Combined Balance Sheets

(Unaudited)
(Dollars in thousands)

		March 31, 2024	December 31, 2023
	Assets		
Current assets:			
Cash and cash equivalents	\$	379,167	210,885
Short-term investments		661,172	793,399
Patient accounts receivable		579,412	489,900
Inventories and supplies		104,279	99,513
Other current assets		235,726	186,373
Total current assets		1,959,756	1,780,070
Investments		1,624,871	1,586,220
Investments with limited uses		344,514	371,147
Beneficial interests in net assets of Allina Foundations		258,577	238,789
Operating lease right-of-use assets, net		116,068	119,570
Land, buildings, and equipment, net		1,326,466	1,324,757
Other assets		165,969	161,126
Total assets	\$	5,796,221	5,581,679
	Liabilities and Net Assets		
Current liabilities:			
Accounts payable and accrued expenses	\$	438,144	481,195
Current portion of long-term debt		21,575	21,575
Current portion of operating lease liabilities		16,615	16,659
Revolving line of credit		380,000	180,000
Other current liabilities		221,644	172,844
Total current liabilities		1,077,978	872,273
Long-term debt		1,674,519	1,676,751
Operating lease liabilities		109,941	113,416
Other liabilities		116,734	113,391
Total liabilities		2,979,172	2,775,831
Net assets:			
Without donor restriction		2,548,923	2,557,774
With donor restriction		268,126	248,074
Total net assets		2,817,049	2,805,848
Total liabilities and net assets	\$	5,796,221	5,581,679

Allina Obligated Group

Combined Statements of Operations and Changes in Net Assets

(Unaudited)
(Dollars in thousands)

	Three Months Ended	
	March 31, 2024	March 31, 2023
Revenues:		
Patient service revenue	\$ 978,184	927,494
Other operating revenue	97,726	77,670
Total revenues	<u>1,075,910</u>	<u>1,005,164</u>
Expenses:		
Salaries and benefits	655,360	667,534
Supplies and services	294,147	287,339
Depreciation and amortization	42,959	44,884
Financing costs	13,848	14,429
Services provided by (to) related divisions	(37,021)	(33,405)
State assessments and taxes	20,685	19,150
Utilities and maintenance	18,543	22,576
Other operating expenses	48,197	42,798
Total expenses	<u>1,056,718</u>	<u>1,065,305</u>
Operating income (loss) before restructuring expenses	19,192	(60,141)
Restructuring expenses	<u>28,866</u>	<u>—</u>
Operating income (loss)	(9,674)	(60,141)
Nonoperating gains (losses):		
Investment return	47,241	82,978
Gain (loss) on interest rate swap agreements	—	(4,314)
Other	(920)	(846)
Excess of revenues over expenses	<u>\$ 36,647</u>	<u>17,677</u>

Allina Obligated Group

Combined Statements of Operations and Changes in Net Assets

(Unaudited)
(Dollars in thousands)

	Three Months Ended	
	March 31,	March 31,
	2024	2023
Net assets without donor restrictions:		
Excess (deficiency) of revenues over expenses	\$ 36,647	17,677
Net assets released from restrictions for capital purposes	—	110
Transfer net assets released to Obligated Group	667	10,512
Capital contributions to nonobligated group affiliates, net	(46,177)	(47,483)
Other	12	39
(Decrease) increase in net assets without donor restrictions	(8,851)	(19,145)
Net assets with donor restrictions:		
Contributions	—	119
Net assets released from restrictions	—	(118)
Change in beneficial interests in net assets of Allina Foundations	19,788	(1,171)
Other	264	213
Increase (decrease) in net assets with donor restrictions	20,052	(957)
Increase (decrease) in net assets	11,201	(20,102)
Net assets at beginning of year	2,805,848	2,960,430
Net assets at end of quarter	\$ 2,817,049	2,940,328

Allina Health System

Consolidating Balance Sheet

(Unaudited)
(Dollars in thousands)
March 31, 2024

Assets	Obligated Group	Other nonobligated	Eliminations	Allina Health System
Current assets:				
Cash and cash equivalents	\$ 379,167	10,960	—	390,127
Short-term investments	661,172	—	—	661,172
Patient accounts receivable	579,412	105,336	—	684,748
Inventories and supplies	104,279	2,223	—	106,502
Other current assets	235,726	40,771	—	276,497
Total current assets	1,959,756	159,290	—	2,119,046
Investments	1,624,871	83,307	—	1,708,178
Investments with limited uses	344,514	190,822	—	535,336
Beneficial interests in net assets of Allina Foundations	258,577	—	(258,577)	—
Operating lease right-of-use assets, net	116,068	95,912	—	211,980
Land, buildings, and equipment, net	1,326,466	188,369	—	1,514,835
Other assets	165,969	139,130	—	305,099
Total assets	\$ 5,796,221	856,830	(258,577)	6,394,474
Liabilities and Net Assets				
Current liabilities:				
Accounts payable and accrued expenses	\$ 438,144	98,283	—	536,427
Current portion of long-term debt	21,575	746	—	22,321
Current portion of operating lease liability	16,615	18,891	—	35,506
Revolving Line of Credit	380,000	—	—	380,000
Other current liabilities	221,644	—	—	221,644
Total current liabilities	1,077,978	117,920	—	1,195,898
Long-term debt	1,674,519	7,525	—	1,682,044
Operating lease liabilities	109,941	92,966	—	202,907
Other liabilities	116,734	82,069	—	198,803
Total liabilities	2,979,172	300,480	—	3,279,652
Net assets:				
Without donor restriction				
Controlling interest	2,548,923	318,657	—	2,867,580
Noncontrolling interest in subsidiaries	—	25,979	—	25,979
Total net assets without donor restrictions	2,548,923	344,636	—	2,893,559
With donor restriction	268,126	211,714	(258,577)	221,263
Total net assets	2,817,049	556,350	(258,577)	3,114,822
Total liabilities and net assets	\$ 5,796,221	856,830	(258,577)	6,394,474

Allina Health System

Consolidating Statement of Operations and Changes in Net Assets

(Unaudited)
(Dollars in thousands)

Three Months ended March 31, 2024

	Obligated Group	Other nonobligated	Eliminations	Allina Health System
Revenues:				
Patient service revenue	\$ 978,184	255,919	—	1,234,103
Other operating revenue	97,726	7,043	8,614	113,383
Total revenues	1,075,910	262,962	8,614	1,347,486
Expenses:				
Salaries and benefits	655,360	196,710	—	852,070
Supplies and services	294,147	45,548	—	339,695
Depreciation and amortization	42,959	8,302	—	51,261
Financing costs	13,848	1,708	—	15,556
Services provided by (to) related divisions	(37,021)	28,407	8,614	—
State assessments and taxes	20,685	4,839	—	25,524
Utilities and maintenance	18,543	1,907	—	20,450
Other operating expenses	48,198	14,205	—	62,403
Total expenses	1,056,719	301,626	8,614	1,366,959
Operating income (loss) before restructuring expenses	19,191	(38,664)	—	(19,473)
Restructuring expenses	28,866	—	—	28,866
Operating income (loss)	(9,675)	(38,664)	—	(48,339)
Nonoperating gains (losses):				
Investment return	47,241	979	—	48,220
Other	(919)	(64)	—	(983)
Excess (deficiency) of revenues over expenses	36,647	(37,749)	—	(1,102)
Less income attributable to noncontrolling interests	—	(679)	—	(679)
Excess (deficiency) of revenues over expenses - attributable to controlling interests	\$ 36,647	(38,428)	—	(1,781)

Allina Health System

Consolidating Statement of Operations and Changes in Net Assets

(Unaudited)
(Dollars in thousands)

Three Months ended March 31, 2024

	Obligated Group	Other nonobligated	Eliminations	Allina Health System
Net assets without donor restrictions, controlling interest:				
Excess (deficiency) of revenues over expenses	\$ 36,647	(38,428)	—	(1,781)
Net assets released from restrictions for capital purposes	—	652	—	652
Transfer net assets released to Obligated Group	667	(667)	—	—
Capital contributions to nonobligated group affiliates, net	(46,177)	46,177	—	—
Other	12	(27)	—	(15)
(Decrease) increase in net assets without donor restrictions, controlling interest	(8,851)	7,707	—	(1,144)
Net assets without donor restrictions, noncontrolling interest:				
Excess (deficiency) of revenues over expenses	—	679	—	679
Distributions to noncontrolling interests	—	(1,031)	—	(1,031)
Other	—	435	—	435
Increase in net assets without donor restrictions, noncontrolling interest	—	83	—	83
Net assets with donor restrictions:				
Contributions	—	7,717	—	7,717
Investment return	—	8,448	—	8,448
Net assets released from restrictions	—	(1,966)	—	(1,966)
Change in beneficial interests in net assets of Allina Foundations	19,788	—	(19,788)	—
Other	264	—	—	264
Increase (decrease) in net assets with donor restrictions	20,052	14,199	(19,788)	14,463
Increase (decrease) in net assets	11,201	21,989	(19,788)	13,402
Net assets at beginning of year	2,805,848	534,361	(238,789)	3,101,420
Net assets at end of quarter	\$ 2,817,049	556,350	(258,577)	3,114,822

Consolidated Utilization

	Three Months Ended		Year Ended	
	March 31	March 31	December 31	December 31
	2024	2023	2023	2022
Hospitals				
Admissions *	24,016	22,480	93,531	89,671
Patient Days *	120,187	118,140	461,286	456,680
Average length of stay	5.0	5.3	4.9	5.1
Observation days	7,886	12,323	45,483	44,255
Licensed beds	2,451	2,451	2,451	2,451
Staffed beds	1,851	1,802	1,851	1,800
Outpatient visits	298,957	297,211	1,218,757	1,170,872
Emergency room visits	68,247	63,466	345,979	327,847
Inpatient surgical procedures	6,513	6,405	26,192	25,545
Outpatient surgical procedures	12,517	13,318	54,075	50,940
Ambulatory Surgery Center procedures	4,374	3,163	14,290	10,465
Total surgical procedures	23,404	22,886	94,557	86,950
Physicians and allied professionals				
Work RVUs	2,703,203	2,574,338	10,469,210	9,938,529
Ambulance transports	22,408	24,416	101,152	97,563

*results exclude newborns

Management's Discussion and Analysis of Results of Operations

Operating Results

Allina Health's operating margin before restructuring expenses was -1.4% for the three months ended March 31, 2024, compared to -8.1% operating margin for the same period in 2023. Allina Health's operating loss improved by \$82.2 million when compared to the same period in 2023.

Allina Health's Earnings Before Interest, Depreciation and Amortization (EBIDA) margin was 3.5% for the three months ended March 31, 2024, compared to -2.6% for the same period in 2023. Non-operating investment return and interest rate swap unrealized activity were consistent with market conditions.

EARNINGS SUMMARY	ALLINA HEALTH		OBLIGATED GROUP	
	Three Months Ended March		Three Months Ended March	
	2024	2023	2024	2023
EBIDA	\$ 46.9	\$ (34.1)	\$ 76.0	(1.4)
Interest income	0.6	1.2	-	0.6
Financing costs	(15.6)	(16.5)	(13.8)	(14.4)
Depreciation and amortization	(51.3)	(52.2)	(43.0)	(44.9)
Operating income(loss) before non-recurring	(19.4)	(101.6)	19.2	(60.1)
Restructuring expenses	(28.9)	-	(28.9)	-
Operating (loss) income	(48.3)	(101.6)	(9.7)	(60.1)
Investment return	48.2	84.1	47.2	83.0
Interest rate swap agreements	-	(4.3)	-	(4.3)
Non-operating other	(1.0)	(0.9)	(0.9)	(0.9)
Excess of revenues over expenses	<u>\$ (1.1)</u>	<u>\$ (22.7)</u>	<u>\$ 36.6</u>	<u>\$ 17.7</u>

Patient Service Revenue

Allina Health's total operating revenue increased \$101.1 million, or 8.1% for the three months ended March 31, 2024, as compared to 2023. Through March 31, 2024, 37.5% of patient revenue is inpatient revenue, compared to 37.4% in 2023.

PATIENT REVENUE	ALLINA HEALTH		OBLIGATED GROUP	
	Three Months Ended March		Three Months Ended March	
	2024	2023	2024	2023
Inpatient	\$ 462.8	\$ 431.5	\$ 464.4	\$ 431.8
Change	7.3%		7.5%	
Outpatient/Ambulatory	442.7	427.2	422.2	411.3
Change	3.6%		2.7%	
Clinic	322.0	290.4	87.6	80.3
Change	10.9%		9.1%	
Other patient revenue	6.6	6.2	4.0	4.1
Change	6.5%		-2.4%	
Total Patient Service Revenue	<u>\$ 1,234.1</u>	<u>\$ 1,155.3</u>	<u>\$ 978.2</u>	<u>\$ 927.5</u>
Change	6.8%		5.5%	

Managed Care Relationships

Allina Health negotiates and contracts with managed care payers through a centralized payer relations function. Managed care payers accounted for 47.3% of Allina Health's total patient service revenues before implicit price concessions for the three months ended March 31, 2024. Allina Health's top three managed care payers comprise 40.6% of Allina Health's revenue. Allina Health's contract with its largest managed care payer (Blue Cross) expires on December 31, 2026. The contracts with Allina Health's next three largest managed care payers have contracts expiring December 31, 2024 through December 31, 2026. Allina Health, separately and in conjunction with managed care payers, continues to redesign clinical processes and to develop evidenced-based guidelines in order to decrease costs and provide the demonstrable improvements in quality required by managed care.

ALLINA HEALTH		Patient Revenue	
		March 31	
PAYER MIX PERCENTAGE		2024	2023
Medicare		38.0%	35.3%
Medicaid		11.2	11.2
Contracted Payers		47.3	49.2
Self Pay		1.3	1.6
Other		2.2	2.7
Total		100.0%	100.0%

Uncompensated Care

Allina Health provides medical care without charge or at reduced cost to patients who live in the communities that it serves through the provision of charity care. Allina Health identifies patients that qualify for charity care based upon certain guidelines related to a patient's ability to pay for services. The Allina Health hospitals provide a discount on billed charges for medically necessary care delivered to patients who are uninsured, underinsured and ineligible for government programs or otherwise medically indigent. New legislation that changed the uninsured discount rate hospitals provide to uninsured patients as well as requiring hospitals and health systems to screen for charity care eligibility was implemented on Nov. 1, 2023. This new legislation, combined with the unwinding of the Public Health Emergency, has led to a significant increase in uninsured discounts.

The estimated cost of providing charity care, by applying a cost to charge ratio to charges identified as charity care, for the three months ended March 31, 2024, and 2023 were \$3.5 million and \$4.3 million, respectively. The schedule below reflects uncompensated care at gross charges forgone.

UNCOMPENSATED CARE	ALLINA HEALTH	
	Three Months Ended	
	March 31	
	2024	2023
Uninsured Discount	\$ 27.8	\$ 12.6
Charity Care Discount	9.5	11.5
Implicit Price Concessions	21.3	23.3
Total Uncompensated Care	\$ 58.6	\$ 47.4
Change	23.6%	
Total Uncompensated Care as a % of		
Patient Charges before price concessions	1.6%	1.4%

Volume

Inpatient admissions increased by 6.8%, and inpatient surgeries increased 1.7% for the three months ended March 31, 2024 compared to 2023, while observation days decreased 36%. Total outpatient surgeries decreased 6.0%, offset by a 38.3% increase of surgeries at the ambulatory surgery centers. Outpatient visits increased 0.6%. Clinic work RVUs increased 5.0%.

VOLUME STATISTICS	ALLINA HEALTH	
	Three Months Ended	
	March 31	
	2024	2023
Inpatient Hospital Admissions	24,016	22,480
Inpatient change from prior period	6.8%	
Observation Days	7,886	12,323
Observation days change from prior period	-36.0%	
Outpatient Visits	298,957	297,211
Outpatient change from prior period	0.6%	
Average Length of Stay (days)	5.0	5.3
Hospital Patient Days	120,187	118,140
Patient days change from prior period	1.7%	
Hospital Occupancy (based on staffed beds)	71.4%	72.0%
Inpatient Surgeries	6,513	6,405
Outpatient Surgeries	12,517	13,318
Ambulatory Surgery Center Surgeries	4,374	3,163
Total Surgeries	23,404	22,886
Total surgeries change from prior period	2.3%	
Clinic Work RVUs	2,703,203	2,574,338
Work RVUs change from prior period	5.0%	
Total Case Mix	1.80	1.74
Medicare Case Mix	1.96	2.06

Market Share

Allina Health continues to hold its market leading position. The following market share data from the Minnesota Hospital Association (MHA) statistical database for the 11-county metropolitan hospital market is updated as of September 30, 2023.

MARKET SHARE STATISTICS	ALLINA HEALTH	
	September 30 2023	December 31 2022
Allina Metro Hospital Inpatient Market Share *	31.1%	30.6%
Change in Total Metro Market Volume**	4.4%	-2.9%
* Hospitals Include: Abbott Northwestern, United, Mercy and St. Francis		
** year over year change of the eleven county metro hospital inpatient market volume for the twelve months ending.		

Non-Operating Gains and Losses

Allina Health investments include a diversified portfolio of money market, fixed income, equity, private capital, and real asset investments. The total return on unrestricted long-term investments was 2.3% for the three months ended March 31, 2024, consistent with the market conditions for the period. To mitigate changes in interest rates on variable rate debt, Allina Health held fixed-payer swaps, which were marked to market prior to terminating the swaps effective June 30, 2023.

NON-OPERATING GAINS & LOSSES	ALLINA HEALTH		OBLIGATED GROUP	
	Three Months Ended March		Three Months Ended March	
	2024	2023	2024	2023
Interest and dividends	\$ 24.3	\$ 16.8	\$ 24.2	\$ 16.8
Realized gains on sales of investments	6.8	0.1	6.6	0.6
Unrealized (losses) gains on investments	17.1	67.2	16.4	65.6
Interest rate swap agreements - fair value	-	(3.5)	-	(3.5)
Interest rate swap agreements - counterparty interest	-	(0.8)	-	(0.8)
Other	(1.0)	(0.9)	(0.9)	(0.9)
Total non-operating	<u>\$ 47.2</u>	<u>\$ 78.9</u>	<u>\$ 46.3</u>	<u>\$ 77.8</u>

Balance Sheet and Cash Flow

Allina Health has a syndicated revolving line of credit in the amount of \$400 million. There was a \$189 million draw on the syndicated revolving line of credit in April 2023 which was used in connection with the redemption of the Series 2007C Bonds, Series 2009B Bonds and Series 2009C Bonds. The draw for redemption of the prior bonds is expected to be repaid by the end of 2024. Allina Health drew an additional \$200 million in March 2024 due to the inability to bill certain claims after the Change Healthcare cyberattack, and paid down that draw in April 2024. The outstanding draw as of March 31, 2024 was \$380 million.

Allina Health had 165 days cash on hand as of December 31, 2023, compared with 174 days at December 31, 2023. Amounts drawn on the line of credit are not included in the days cash on hand calculation.

Leverage was 37.1% as of March 31, 2024 and December 31, 2023. Cash to debt is 140% as of March 31, 2024, compared to 147% at December 31, 2023.

The Obligated Group historical annual debt service coverage ratio is calculated in accordance with the Master Trust Indenture on a twelve month look back and adjusted for nonrecurring restructuring expenses for March 31, 2024.

Allina Health capital spending was \$57 million for the three months ended March 31, 2024, compared to capital spending of \$47.7 million for the same time period in 2023.

BALANCE SHEET & LIQUIDITY STATISTICS	ALLINA HEALTH		OBLIGATED GROUP	
	March 2024	December 31 2023	March 2024	December 31 2023
Unrestricted cash & investments	\$ 2,759.5	\$ 2,688.0	\$ 2,665.2	\$ 2,590.5
Days cash on hand	165	174	202	212
Total Debt	\$ 1,704.4	\$ 1,707.7	\$ 1,696.1	\$ 1,698.3
Net assets without donor restrictions	\$ 2,893.6	\$ 2,894.6	\$ 2,548.9	\$ 2,557.8
Debt to capitalization *	37.1%	37.1%	40.9%	40.9%
Patient receivables	\$ 684.7	\$ 580.3	\$ 579.4	\$ 489.9
Days revenue in receivables, net	52	45	55	38
Cash to debt	140%	147%	135%	142%
Historical annual debt service coverage			3.7	5.8

*Obligated Group includes Letters of Credit and Surety indebtedness

Debt and Investment Appendix

Debt Structure

Allina Health holds a mix of both fixed and variable rate long-term debt. The following chart displays the structure and provides detail regarding credit enhancement and expiration of the credit agreement, if applicable.

Allina Health's current debt structure as of March 31, 2024					
Series	\$ Outstanding	Structure	Maturity	Credit Enhancement	Int. Rate*
2023A	181,500	Fixed Rate	2052	None	5.00%
2023B	181,500	Fixed Rate	2053	None	5.00%
2021 Taxable	302,323	Fixed Rate	2051	None	2.90%
2021	167,770	Fixed Rate	2040	None	4.00%
2019 Taxable	300,000	Fixed Rate	2049	None	3.89%
2019	50,525	Fixed Rate	2029	None	5.00%
2017 Taxable	150,000	Fixed Rate	2047	None	4.43%
2017A	58,500	Fixed Rate	2029	None	5.00%
2015 Taxable	250,000	Fixed Rate	2045	None	4.81%
	<u>\$ 1,642,118</u>	Total Bonds			
Other **	<u>62,247</u>				
	\$ 1,704,365	Total Debt			
Fixed Rate		\$ 1,642,118	96.3%		
Other		<u>62,247</u>	3.7%		
Total		\$ 1,704,365			

*Interest rates are interest cost only, and do not include premium/discount, administrative, credit facility, broker, or other costs related to the issuance of the bonds.

**Other debt includes unamortized deferred financing costs, premiums or discounts associated with fixed rate debt and other small notes and loans that are included in debt on the balance sheet.

Investment Pools

The Investment Committee, a committee of the Allina Health Board, is responsible for overseeing asset allocation studies; evaluating and selecting an independent investment consultant; developing investment objectives, guidelines, and performance measurement standards; evaluating and selecting investment managers; and reviewing and evaluating the results of each investment component and the overall performance of all invested funds.

Allina Health maintains its unrestricted investments in cash, cash equivalents and short-term fixed income (liquidity assets), which are utilized for liquidity and preservation of capital, and diversified long term investments (long-term assets), which are utilized for capital growth. The allocation between liquidity and long-term assets depends on the liquidity and strategic needs of the organization. The following table allocates assets based on investment strategy, and will vary from the audited footnote, which looks through the investment strategies to the underlying holdings.

Allina Health periodically reviews asset allocation to ensure that the organization is maintaining the appropriate liquidity, portfolio allocation, and to address shifts in market expectations. The Investment Committee of the Allina Health Board approved a new long-term asset allocation target in 2021, which is reflected in the following table. The Investment Committee determined it was appropriate to temporarily increase the liquidity in the portfolio in the fourth quarter of 2023, without adjusting the long-term asset allocation targets.

Unrestricted Cash and Investments - Asset Allocation					
Unrestricted Balances	Current Target	3/31/2024		3/31/2023	
Cash and Cash Equivalents		\$	390,127	\$	135,265
Short-Term Fixed Income	15.0%	27.9%	661,172	14.1%	374,627
Long-Term Assets	85.0%	72.1%	1,708,178	85.9%	2,284,179
		100.0%	2,369,350	100.0%	2,658,806
Total Unrestricted Assets		\$	2,759,477	\$	2,794,071
Asset Allocation - Asset Class					
Investment	Current Target Allocation	3/31/2024 % of Long Term Assets	3/31/2024 % Total Unrestricted Investments	3/31/2023 % of Long Term Assets	3/31/2023 % Total Unrestricted Investments
Global Equity	36.0%	22.4%	16.1%	37.4%	32.1%
Long/Short Equity Hedge Funds	0.0%	0.1%	0.1%	0.4%	0.3%
Global Fixed Income	46.0%	53.5%	38.6%	39.8%	34.2%
Alternatives	18.0%	23.9%	17.2%	22.2%	19.1%
Real Return	0.0%	0.1%	0.1%	0.2%	0.2%
Total Long-Term	100.0%	100.0%	72.1%	100.0%	85.9%

Summary of Key Financial Ratios

The table below is a summary of key financial ratios.

Indicators	Three Months Ended March 31,		Year Ended December 31,	
	2024	2023	2023	2022
Liquidity Ratios:				
Monthly DCOH*	140	175	143	193
Annual DCOH**	140	178	143	200
Traditional DCOH	165	204	174	226
Days Cash on Hand (Obligated Group)	202	211	212	222
Cash to Debt	140%	173%	147%	186%
Days Revenue in Receivables	52	46	45	46
VRDB Debt (in millions)	\$0	\$279	\$0	\$279
Monthly liquidity to VRDB debt	-	857%	-	920%
Capital Structure Ratios:				
Historical Coverage Ratio (x) (Obligated Group)	3.7	1.8	5.8	2.1
Cushion Ratio (x) (Obligated Group)	31.4	29.1	77.2	35.9
Leverage (Obligated Group)	40.9%	38.0%	40.9%	37.8%
Leverage	37.1%	34.8%	37.0%	34.7%
Profitability Ratios:				
EBIDA Margin	3.5%	-2.6%	-1.0%	1.8%
Operating Margin	-1.4%	-8.1%	-6.2%	-3.4%
Net Income Margin	-0.1%	-1.8%	-3.2%	-8.0%
Revenue Growth	8.1%	6.2%	5.5%	0.6%

Note: All ratios are for Allina Health consolidated financial results, unless otherwise noted.
All ratios exclude restructuring and strike expenses, where applicable.
DCOH excludes draws on revolving lines of credit and proceeds on 364-day Term Loan.

* Days cash on hand available within 0 – 30 days

**Days cash on hand available within 0 – 365 days